

Policies to ensure the sustainability of long-term care funding

Overview of needs assessments and eligibility across OECD countries

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Information Note: This is a preliminary report on needs assessments and eligibility for the project. Additional countries will be included in a second version as well as more tables and a more complete description of the findings.

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1. Eligibility criteria to determine access to long-term care across countries

1. Countries use eligibility criteria to determine whether a person qualifies for long-term care (LTC) benefits to ensure that limited resources are allocated effectively and equitably to those who need them the most. In most countries, these criteria concern income- and asset-testing, age limits, a person's living situation, their work or insurance status or residency and most importantly the needs assessment (described in the next section) to decide on a person's eligibility for LTC services or the share of the LTC cost covered by public support.

2. The focus on age as a criterion for LTC benefits varies considerably across different countries, reflecting diverse approaches to LTC (Table 1). Some countries explicitly design their programmes to cater to individuals over a certain age, typically 60 or 65, recognising the increasing likelihood of dependency and care needs as people age. This age-specific focus ensures that older adults receive the necessary support, particularly as they transition into retirement. For example, Australia, Japan, and Korea have clear age thresholds for eligibility, often requiring individuals to be at least 65 years old to access LTC services. Conversely, there are countries where an age criterion is not specified or all age groups are included, though they often include special provisions for older adults like in Austria, Canada, Czechia, Estonia, and Finland. However, even within these systems, older adults often form a significant portion of the beneficiaries, given their higher likelihood of requiring LTC. For example, in Austria and Finland, LTC benefits are available to all residents regardless of age, but older people are typically the primary recipients due to their higher incidence of chronic conditions and disabilities.

3. Another significant area of variation lies in the minimum level of dependency required for individuals to qualify for long-term benefits. In some countries, eligibility hinges on substantial and ongoing care needs. For instance, Austria requires that an individual needs care for more than 65 hours per month for at least six months. In Germany, eligibility for the *Pflegegeld* (Care Allowance) begins at Care Level 2, which involves considerable impairments in self-sufficiency. Similarly, in Spain, individuals must be classified as having Grade II or III of dependency to qualify for residential care services. In other countries, the criteria are slightly less stringent but still require significant care needs. In Canada (Ontario), for instance, individuals must demonstrate that they require 24-hour care that cannot be met through community-based services to qualify for LTC homes. In the United Kingdom, eligibility for social care or attendance allowances is based on a physical or mental disability that impairs two or more functional capacities for at least six months. Finland's system also requires a diagnosis of illness or disability, with a documented impairment of functional capacity for at least a year.

4. Almost all LTC schemes require a needs assessment as a prerequisite to receive benefits. The few exceptions are complementary services in France and Belgium, such as service vouchers for daily shopping and transport or social housing support for social assistance recipients. A range of countries also require individuals to be affiliated with the local or national insurance scheme in order to be eligible to the benefit schemes. This is the case in e.g. Belgium, Ontario, Luxembourg, the Netherlands, New Zealand, and California. Two countries require the individuals to be pensioners: Finland and Portugal. Similarly, a residence status is not required in most countries. Those who specify one, set it at a minimum of 2 (UK) to 5 years (Belgium, Italy, and Spain).

Table 1. Eligibility criteria for access to LTC benefits

Countries and subnational areas	Benefit and schemes	Benefit type	Needs assessment required	Min dependency level / Additional criteria	Age (years)	Residency	Work/Insurance status
Australia	Commonwealth Home Support Programme Home Care Package Aged Care Home	In kind	Yes	Not specified	> 65 ¹	All residents	Not specified
Austria (Viena)	Care allowance (Pflegegeld)	Cash	Yes	Care required for more than 65 hours a month and for at least 6 month or more.	Not specified	All residents	Not specified
	24-hour care	Cash	Yes	Pflegegeld beneficiaries on level 3 and above (>120 hours of care a month).	Not specified	All residents	Not specified
	Fonds Soziales Wein	In-kind	Yes	Not specified	Not specified	All residents	Not specified
Belgium (Flanders)	Care budget for people with severe care needs	Cash	Yes	At least one of the following degrees: * at least B score on the Katz scale in a care institution; * a total score of at least 13, or 5.5 points at least for the sum of the IADL and ADL modules of the BelRAI Screener, taken within the framework of the care budget (indication) or within the framework of the social investigation for the granting of family care or the additional home care; * a score of at least 15 on the medical-social scale (with the view of the review of the entitlement to the integration allowance, to the budget care for elderly persons requiring care or to the assistance allowance for elderly persons); * score C or Cd on the Katz scale in a care institutions; * certificate for palliative care allowance or palliative flat-rate amount: only valid for a first application for the care budget for severely dependent persons; * Kine-E: only valid if you have received, for three years, a positive decision based on an indication with the BEL scale or the BelRAI Screener.	None	Resident for > 5 consecutive years ²	Affiliation to the Flemish social protection system.

Countries and subnational areas	Benefit and schemes	Benefit type	Needs assessment required	Min dependency level / Additional criteria	Age (years)	Residency	Work/Insurance status
	Care budget for the elderly with care needs	Cash	Yes	If living at home, score at least 7 points on the medico-social scale.	> 65	Resident for > 5 consecutive years ²	Affiliation to the Flemish social protection system.
	Allowance for the chronically ill	Cash	No	* Spent > EUR 450 per year (without increased reimbursement for chronically ill), AND * Receiving physiotherapy or physical therapy in relation to a serious condition for at least 6 months; or being admitted to hospital at least 6 times or for at least 120 days within a calendar year.	None	Not specified	Not specified
	Incontinence allowance	Cash	Yes	B or C on the Katz scale and score 3 or 4 in the dependency schedule for the criterion "incontinence".	None	Not specified	Not specified
	Service vouchers	In-kind	No	None, but daily shopping and transport activities are offered to beneficiaries with limited mobility.	> 18	All residents	Not specified
	Home care organisations	In-kind	Yes	Not specified	None	All residents	Not specified
Canada (Ontario)	Home and Community Care	In-kind	Yes	Not specified	All ages	All residents	Insured under Ontario Health Insurance Plan
	Long-term Care Homes	In-kind	Yes	24 hours care required that cannot be met through community-based services.	> 18	All residents	Insured under Ontario Health Insurance Plan
Czechia	Care allowance (Příspěvek na péči)	Cash	Yes	Unable to perform at least 3 out of 10 basic living needs.	> 1	All residents	Not specified
Denmark	Home help (Hjemmehjælp) Nursing Home Housing (Plejehjemsboliger)	In-kind	Yes	Not specified	Not specified	All residents	Not specified
Estonia (Tallinn)	Domestic care service (koduteenus)	In-kind	Yes	Not specified	> 18	All residents	Not specified
	24/7 general care (Ööpäevane üldhooldus)	In-kind	Yes	24 hours care required that cannot be met through community-based services.	Not specified	All residents	Not specified

Countries and subnational areas	Benefit and schemes	Benefit type	Needs assessment required	Min dependency level / Additional criteria	Age (years)	Residency	Work/Insurance status
Finland	Care allowance (Eläketä saavan hoitotuki)	Cash	Yes	Illness or disability diagnosed by a doctor and functional capacity impaired for at least a year.	Retirement age ³	All residents	Retired full-time ³
	Home care services (Kotihoito and Tukipalvelut) Institutional care	In-kind	Yes	Not specified	Retirement age ³	All residents	Not specified
France	Allocation Personnalisée d'Autonomie (APA)	Cash	Yes	Being classified as GIR 1, 2, 3, or 4.	> 60	All residents	Not specified
	Aide sociale à l'hébergement (ASH)	Both	No	Must reside in an establishment which has places reserved for social assistance recipients, or with a host family. Income must be less than the accommodation fees charged by the establishment.	> 65 ⁴	All residents	Not specified
	Personne âgée : aide financière pour rémunérer une aide à domicile	Both	Yes	Not qualified for APA, and difficulty performing basic household tasks (e.g., cleaning home, doing laundry and meal preparation).	> 65 ⁴	All residents	Not specified
	Targeted tax reductions	Tax credit	No	Only for paid home help or nursing home.	None	All residents	None
Germany	Pflegegeld (Care Allowance)	Cash	Yes	Care level 2 onwards (considerable impairments of self-sufficiency or of skills)	None	All residents	Not specified
	Pflegesachleistungen (Care Benefits In-Kind)	In-kind					
	Institutional care	In-kind					
Ireland	Home support service	Both	Yes	Not specified	> 65 ⁵	All residents	Not specified
	Nursing home support scheme	In-kind	Yes	24 hours care required that cannot be met through community-based services.	None	All residents	Not specified
Israel	Home care	Both	Yes	Qualify for level 1 out of 6	Retirement age (67 for men, 62-65 for women)	All residents	Not specified
Italy (South Tyrol)	Care allowance (Assegno di cura)	Both	Yes	Functional limitations expected to last more than 6 months, or limitations that have already been present for more than 6 months at the time of application.	None	Residents for >5 years ⁶	Not specified
	Home care services (Assistenza domiciliare) Residential services	In-kind	Yes	Not specified	None	Not specified	Not specified
Japan	Long-term Insurance System (介護保険)	In-kind	Yes	Not specified	> 65 ⁷	Not specified	Not specified

Countries and subnational areas	Benefit and schemes	Benefit type	Needs assessment required	Min dependency level / Additional criteria	Age (years)	Residency	Work/Insurance status
	制度)						
Korea	Long-Term Care Insurance for the Elderly (노인장기요양보험제도)	Both	Yes	Classified at one of the six levels of need severity.	> 65 ⁸	Not specified	Not specified
Lithuania	Social care Social assistance Institutional care Municipal support	In-kind	Yes	Not specified	None	All residents	Not specified
		Cash	Yes	Not specified	None	All residents	Not specified
Luxembourg	Long-term care insurance	Both	Yes	Care required for basic everyday activities for at least 3.5 hours per week and the condition is likely to last longer than 6 months or to be irreversible.	None	All residents	Affiliated to the National Health Fund.
	Complément accueil g�rontologique	In-kind	Yes	Long-term care insurance beneficiaries with an indefinite admission to an integrated centre for the elderly or a care home.	Not specified	All residents	Not specified
Netherlands	Social Support Act 2015 (Wet Maatschappelijke Ondersteuning 2015, Wmo)	Both	Yes	Not specified	None	All residents	None
	Health Insurance Act (Zorgverzekeringswet, Zvw)	Both	Yes	Not specified	None	All residents	Covered by the compulsory basic insurance "Basic care package"
	Long-Term Care Act (Wet Langdurige Zorg, WLZ)	Both	Yes	Qualifying period of one month for every year that the person is uninsured up to a max of 12 months.	None	All residents	Insured under Health Insurance Act
New Zealand	Home support services	In-kind	Yes	Not specified	> 65 ⁹	All residents	Community Services Card holder for some services.
	Residential care	In-kind	Yes	Not specified	> 65 ¹⁰	All residents	Not specified
Poland	Zasilek pielęgnacyjny	Cash	Yes	None for > 75 aged individuals.	> 75 ¹¹	All residents	Not specified
	Care supplement (dodatek pielęgnacyjny)	Cash	Yes ¹²	None	> 75 ¹³	All residents	Not specified
	Home care services (usługi opiekuńcze w miejscu zamieszkania)	In-kind	Yes	Not specified	None	All residents	Not specified
	Residential care	In-kind	Yes	Score 40 points or less in the Barthel scale	None	All residents	Not specified
Portugal	Complement by dependency (for home care)	Cash	Yes	Not specified	Retirement age	All residents	Pensioner

Countries and subnational areas	Benefit and schemes	Benefit type	Needs assessment required	Min dependency level / Additional criteria	Age (years)	Residency	Work/Insurance status
	Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	In-kind	Yes	Not specified	> 65 ¹⁴	All residents	Not specified
Slovenia	Assistance and Attendance Allowance (dodatek za pomoč in postrežbo)	Cash	Yes	At least one of the following degrees: * in urgent need of assistance and attendance in order to meet all basic living needs; * in urgent need of assistance and attendance in order to meet the majority of basic living needs; * blind or partially sighted; * having a reduced ability to move of at least 70%.	Retirement age	All residents	Not specified
	Home help [pomoč na domu] (Municipality subsidised care)	In-kind	Yes	Not specified	> 65	All residents	Not specified
Spain	Ayuda al domicilio	In-kind	Yes	- Requires help at least once a day to carry out the most essential daily activities.	None	Residents for > 5 years ¹⁵	None
	Prestación económica vinculada al servicio Prestación económica de asistencia personal	Cash	Yes	'Prestación económica vinculada al servicio', is provided only when access to a public or contracted care service is not possible.			
	Atención Residencial	In-kind	Yes	Grade II and III of dependency required.			
Sweden	Home care (hemtjänst) Institutional care (särskilt boende)	In-kind	Yes		None ¹⁶	All residents	Not specified
United Kingdom	Attendance allowance	Cash	Yes	Physical/mental disability and functional capacity impaired for at least 6 months.	State pension age	Residents for >2 years of the last 3 ¹⁷	Not specified
	Social care	Both	Yes	Physical/mental disability which limits two or more functional capacity.	> 18	Not specified	Nos specified
United States (California)	In-Home Supportive Services (IHSS) Medi-Cal Institutional Care	In-kind	Yes		> 65	None ¹⁸	Medi-Cal insured

Note:

1. 50 years or older if beneficiary identifies as an Aboriginal or Torres Strait Islander person.
2. It does not apply to under 18, and it does not need to immediately precede to the application.
3. A person aged 16 or older receiving old-age pension or related benefit.
4. 60 years or older if declared unfit for work.
5. Exception made for younger individuals (e.g., early onset dementia or disability).
6. Applicant may still qualify if beneficiary has a historical residency of 15 years in the province, with at least one year immediately preceding the application. Stable living arrangements in the province for at least one years is required.

7. Aged 40-64 years who have terminal cancer or geriatric diseases.
8. 65 years or younger if they have a geriatric disease (dementia, cerebrovascular disease and Parkinson's).
9. 55 years or older if beneficiaries identifies as Māori, or 60 years or older with aged-related disabilities.
10. 50 to 64 years if unmarried and with no dependent children.
11. A disabled child or over the age of 16, if they have a certificate of significant disability.
12. Applicants over 75 years of age, the benefit is granted ex officio by the pension authority.
13. A person entitled to a retirement or disability pension if they have been deemed to be totally incapable of work and independent living.
14. Over 60 if applying for foster care.
15. At least 5 years of residence in Spain, 2 of which must immediately precede the date of submission of the application.
16. No age conditions, but the majority of those in need of long-term care are older persons, over the age of 65.
17. It does not apply to refugees or those who have humanitarian protection status.
18. From January 2024, no immigration status required.

Source: OECD analyses based on Long-Term Care Social Protection questionnaire.

Means-testing is widely used to target public support

5. In addition to the eligibility criteria described above, most countries apply some form of means-tested LTC benefits for home and institutional care (either income-testing only, asset-testing only, or both income- and asset-testing). The exact form of means-testing varies widely. Income-testing tends to be the most common approach, either on its own or in conjunction with asset-testing, which is often added to create a more comprehensive means-test. This combined approach can ensure that individuals with high income or significant assets who could pay for care from their means are not using resources which could be targeted at those with less means for care. The use of these methods is summarised in Table 2, which outlines the different approaches across various countries, highlighting the prevalence of these testing mechanisms within their LTC systems.

6. One notable observation is that home care support benefits primarily rely on income-testing alone, while institutional care benefits typically involve both methods (income- and asset-testing, referred as means-testing). This distinction can be attributed to several factors related to the nature and funding of different care settings. Home care services often aim to support individuals in maintaining their independence and quality of life in familiar surroundings. As such, the financial assistance required may be less extensive, allowing income alone to serve as a sufficient criterion. In contrast, institutional care, which includes services provided in nursing homes, usually entails significantly higher expenses due to the comprehensive nature of care required. The costs associated with providing round-the-clock medical supervision, personal care, and accommodations necessitate a more in-depth financial assessment. This might be also based on the assumption that individuals might no longer need their most often possessed asset, which is the house, since they will move to a residence. To avoid that other family members are not left destitute, many countries make exceptions to housing as an asset if the spouse, for instance, is still using it or do not take primary residence into the wealth-testing.

7. Moreover, this approach may also reflect broader societal values regarding care and support. For instance, by implementing stricter financial assessments for institutional care, countries might be signalling a commitment to safeguarding public resources while encouraging individuals to seek home care solutions when feasible, thereby promoting autonomy and reducing reliance on costly institutional settings whenever possible.

Table 2. Means-testing used for formal long-term care benefits and schemes in the OECD.

Countries and subnational areas	Benefits and schemes	Income-tested	Wealth-tested
Australia	Commonwealth Home Support Programme	No	No
	Home Care Package	Yes	No
	Aged care home	Yes	Yes
Austria (Vienna)	Pflegegeld (Care Allowance)	No	No
	24-Stunden-Betreuung (24-hour care)	Yes	No
	Fonds Soziales Wien (Vienna Social Funds)	Yes	No
Belgium (Flanders)	Care budget for people with severe care needs	No	No
	Care budget for the elderly with care needs	No	No
	Allowance for the chronically ill	No	No
	Incontinence allowance	No	No
	Service vouchers	No	No
	Home care organisations	Yes	No
Canada (Ontario)	Home and Community Care Support Services (home care)	No	No
	Home and Community Care Support Services (institutional care)	Yes	No
Croatia	Subsidised home care for low income	Yes	Yes
	Allowance for assistance and care	Yes	Yes
	Personal disability allowance	Yes	No

Countries and subnational areas	Benefits and schemes	Income-tested	Wealth-tested
Czechia	Care allowance	No ¹	No
Denmark	Hjemmehjælp (Home Help)	No	No
	Plekehjemsboliger (Nursing Home Housing)	No	No
Estonia (Tallinn)	Domestic care service	Yes	No
	Institutional care	Yes	Yes
Finland	Home care services	Yes	No
	Care allowance	No	No
	Informal care benefit	No	No
France	Allocation Personnalisée d'Autonomie	Yes	Yes
	Aide sociale à l'hébergement	Yes	Yes
	Targeted tax reductions	Yes ²	No
Germany*	Pflegegeld (Care Allowance)	No	No
	Pflegesachleistungen (Care Benefits In-Kind)	No	No
	Hilfe zur Pflege (Assistance for Care)	Yes	Yes
Greece	Subsidies to chronically ill patients (for home care)	Yes	No
	Subsidies to chronically ill patients (for institutional care)	Yes	Yes
Hungary	Assistance at home	Yes	No
	Homes for the elderly	Yes	Yes
Iceland (Reykjavik)	Social home service	Yes	No
	Nursing home	No ³	No
Ireland	Home support service	No	No
	Nursing home support scheme	Yes	Yes
Israel	Long-Term Care Insurance	Yes	No
Italy (South Tyrol)	Care allowance	No	No
	Home care services	Yes	Yes
	Residential services	Yes	Yes
Japan	介護保険制度 (Long-Term Care Insurance System)	Yes	Yes ⁴
Korea	노인장기요양보험제도 (Long-Term Care Insurance for the Elderly)	Yes	Yes
Latvia	Home care services	Yes	No
	Care allowance	Yes	No
	Institutional care	Yes	No
	State social maintenance benefit	No	No
Lithuania	Social care	Yes	No
	Social assistance	Yes	No
	Institutional care	Yes	Yes
	Municipal support	Yes	Yes
Luxembourg	Long-term care insurance	No	No
	Complément accueil gérontologique	Yes	No ⁴
Malta	Commcare	No	No
	HomeHelp	No	No
	Carer at home	No	No
	Institutional care	Yes	No
Netherlands	Wet langdurige zorg (Wlz)	Yes	Yes
	Zorgverzekeringswet (Zvw)	No	No
	Wet Maatschappelijke Ondersteuning (Wmo)	No	No
New Zealand	Home support services	Yes ⁵	Yes ⁵
	Residential care	Yes	Yes
Poland	Zasilek pielęgnacyjny	No	No
	Care supplement	No	No
	Home care services	Yes	No
	Residential care	Yes	No
Portugal	Complement por Dependência (for home care)	No	No
	Estrutura residencial para pessoas idosas (for institutional care)	Yes	No

Countries and subnational areas	Benefits and schemes	Income-tested	Wealth-tested
Slovak Republic	Compensation allowances	No	No
	Institutional care	Yes	No
Slovenia	Municipality-subsidised care	Yes	Yes
	Attendance allowance	No	No
Spain	Ayuda al domicilio	Yes	Yes
	Atención Residencial	Yes	Yes
	Prestación económica vinculada al servicio	Yes	Yes
	Prestación económica de asistencia personal	Yes	Yes
Sweden	Home care	Yes	No
	Institutional care	Yes	No
United Kingdom (England)	Attendance allowance	No	No
	Social care	Yes	Yes
United States (California)	In-Home Supportive Services (IHSS)	Yes	No
	Medi-Cal Institutional Care	Yes	No
United States (Illinois)	Home and Community Based Services Waiver	Yes	Yes
	Illinois Medicaid Institutional Care	Yes	Yes

Note: Benefits and schemes that are not applicable to the typical cases of LTC needs used in this report are not included in this table. Benefits that are marked as income- or assets-tested, if income- and/or assets-tested are taken into account for at least one of the typical cases of LTC needs in this report. * In Germany, means-testing is used for those who cannot afford the private co-payments, but it is not applied to the long-term care services themselves. ¹ In France, tax reductions depend on income. ² In Iceland, there is no income-testing for institutional care but a specific ceiling is placed on out-of-pocket costs. ³ In Japan, asset-testing is applied to determine the eligibility of food and accommodation fee reductions at nursing homes. ⁴ Although the benefit is not asset-tested when it is given in Luxembourg, all sums paid through this benefit can be recovered from the care recipient's estate upon death. ⁵ Income and asset testing does not apply to 'personal care' services.

Source: OECD analyses based on the OECD Long-Term Care Social Protection questionnaire.

Income-testing varies across countries

8. Countries adopt varying strategies for income-testing or means-testing (which includes both income and assets) (see Table 2). These strategies range from defining thresholds for maximum co-payments based on a person's income to making the amount of benefits directly dependent on income. The variation in approaches across countries reflects their differing priorities in balancing equitable access with the need for financial sustainability in their LTC systems.

9. A common approach in LTC funding requires care recipients to contribute a certain percentage of their income to the cost of LTC services, particularly for institutional care. This ensures that individuals contribute to their care based on their financial capacity, while public funds cover the remaining expenses. For instance, Finland requires that up to 85% of a care recipient's income be directed towards covering home care or institutional care costs for those with severe LTC needs. Similarly, Ireland requires 80%, while Portugal and Spain ask for up to 90% of income to be allocated to institutional care services. To protect individuals from financial hardship, countries, including Austria, Canada, Finland, Italy, Luxembourg, New Zealand, Slovenia, Spain, Sweden, UK and USA, ensure that a portion of the care recipient's income is reserved for personal use, known as the personal needs reserve. This reserve allows individuals to retain a modest amount of income to cover personal expenses, such as clothing, prescription drugs, or other discretionary needs.

10. Many countries establish income thresholds below which care recipients are eligible for public support, or for higher levels of support, to protect economically vulnerable individuals from excessive financial burdens (Table 3). These thresholds help ensure that those at risk of poverty or below the income poverty line are not required to contribute to the costs of their care, preventing further financial hardship. Table 3 provides an overview of the LTC benefit thresholds used by each country, which can be only income dependant or income and asset dependant.

Table 3. LTC benefit mechanism and income caps

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
Australia	Home Care Package	In-kind	Only applies to income-tested care fee: ≤ AUD 33 027.80 per year: users do not pay this fee. > AUD 33 027.80 per year: up to AUD 18.30 per day. > AUD 63 559.60 per year: AUD 18.30 - 36.60 per day. Thresholds for users with a partner vary. Annual cap for income-test. fee: AUD 33 309.29 Lifetime cap for income-test. fee: AUD 79 942.44	Budget for home services is composed of government and client contribution. Government contribution is a subsidy paid directly to providers, and the amount varies depending on HCP level and supplements if the user has special care needs. If the user is eligible to pay income-tested care fee, the subsidy is reduced by this amount. Client contribution is made up of basic daily fee, income-tested care fees, and additional fees: - Basic daily fee¹: flat fee set by the government at a percentage (15.68 - 17.50%) of the single person rate of the basic age pension, which varies depending on the HCP level (Level 1, 15.68%, AUD 11.43). - Income-tested care fee: based on users' income. Full-time pensioners do not pay this fee. - Additional fees: for any extra services contracted.	-
	Aged care home	In-kind	Means-tested care fee: Income: < AUD 33 027.80 per year Assets: < AUD 59 500 Accommodation cost paid by the government if: Income < AUD 33 027.80 per year and assets < AUD 59 500.00 Accommodation cost paid in full by the applicant if: Income > AUD 82 633.72 per year or assets > AUD 201 231.20 Thresholds for users with a partner vary. ²Annual cap for means-test. fee: AUD 33 309.29 ²Lifetime cap for means-test. fee: AUD 79 942.44	Budget is composed of government and client contribution. Government contribution is a subsidy paid directly to providers, and the amount is based on user's care needs. If the user is eligible to pay means-tested care fee, the subsidy is reduced by this amount. Client contribution is made up of: - Basic daily fee: flat fee set at 85% of the single person rate of the basic age pension (AUD 61.96). - Means-tested care fee: based on client's income and assets. The fee may vary from AUD 0 to 417.41 per day. - Accommodation costs: The room price.	Variable financial hardship assistance available

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
Austria	24-hour care	Cash	Income < EUR 2 500.00* per month in order to receive the benefit. * Increases with dependent family members.	Cash benefit, maximum amount provided by the government is EUR 400 per month for self-employed carers or EUR 800 per month for employed carer. Valued per carer, user can have up to two carers.	-
	Fonds Soziales Wien	In-kind	EUR 626/month	Cost of the home help is subsidised, and the user only contributes with a fee based on their income (including benefits). Max hourly co-payment rates exits, e.g., EUR 24.95 for ADL, EUR 19.00 for IADL, or EUR 12.35 for laundry. For residential care, users pay up to 80% of their net income and the majority of Pflegegeld.	* 20% of user's income, * Special payments such 13 th and 14 th monthly payments in full, * 10% of Pflegegeld level 3 per month (regardless of the level they are on).
Belgium	Care budget for the elderly with care needs	Cash	For single persons (Category A): EUR 17 248.56 per year For cohabiting individuals (Category B): EUR 21 553.53 per year	The benefit is based on both family composition (single or cohabiting) and the care category (ranging from 1 to 5). When calculating the care budget, all income of the applicant and their partner (if applicable) is considered. If the total income exceeds the exemption thresholds, the care budget is reduced by the amount of the excess. The final care budget is determined by the care category, and any excess income will lower the budget accordingly. If the excess income surpasses the maximum budget for the assigned care category, the benefit is reduced to zero.	-
	Service vouchers	In-kind	N/A	Household-related services at a subsidised rate, one voucher is equivalent to one full hour of work. A user can order a max of 500 vouchers per calendar year at EUR 9.00 per voucher for the first 400, and EUR 10.00 per voucher for the remainder. A family can order a max of 1 000 vouchers per calendar year at EUR 9.00 per voucher for the first 800, and EUR 10.00 per voucher for the remainder.	-
	Home care organisations	In-Kind	TBD	User contribution is calculated depending on the composition of the family, and the income of the user and the people living with them in the same home. Discounts apply in cases such as needing heavy care, and home care	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
				provided for 60 hours or more per month.	
Canada (Ontario)	Home and Community Care	In-kind	N/A	Services are funded by the Ontario Ministry of Health and Long-term Care to cover for personal support services (ADLs), homemaking (IADLs) if it is part of personal support services, nursing care and therapeutic services. A maximum of 120 hours per 30-day period can be provided for personal support services. There are exemptions for users with complex care.	-
	Long-term Care Homes	In-kind	Long-Term Care Rate Reduction Program: User may qualify if income < CAD 26 224.00 per year	Government pays for personal and nursing care; user pays for accommodation charges (ie. room and board). Cost varies depending on the room type. Max monthly co-payment fees are: - Basic room : CAD 2 036.40 - Semi-private room: CAD 2 455.24 - Private room: CAD 2 909.36 Long-Term Care Rate Reduction Program is available to aid paying accommodation fees for users in basic room or semi-private room (if spouses/partners living together).	CAD 149 per month (as part of the rate reduction program)
Denmark	Hjemmehjælp (Home Help)	Both	N/A	No cost sharing. Personal and practical assistance is free of charge.	-
	Nursing Home Housing (Plejehjemsboliger)	In-kind	N/A	User pays for rent, food, medicines, electricity and heating. Housing benefits (boligstøtte) exists to pay the rent of nursing homes.	-
Estonia (Tallinn)	Domestic care service (koduteenus)	In-kind	Income < EUR 360 per month for no contribution	Co-payment fee based on user's actual net income and the income limit set by the government. Co-payment thresholds based on income exceeding EUR 360 per month by: EUR 1–150: service fee EUR 2 /hour EUR 151–300: service fee EUR 4 /hour EUR 301–450: service fee EUR 6 /hour EUR 451 or more: service fee EUR 10 /hour	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
	24/7 general care (Ööpäevane üldhooldus)	In-kind	Income < EUR 710.70 per month for assistance to pay co-payment fee.	Cost is composed by: - care component (paid by the government) up to EUR 734 per month. - Co-payment fee. Government may assist with the co-payment fee by covering the difference between the user income and the average old-age pension (EUR 710.70).	-
Finland	Home care services Institutional care	In-kind	N/A	Users pay up to 85% of their income after specific deductions.	EUR 110 -164 depending on service
France	Allocation Personnalisée d'Autonomie (APA) for home help	Cash	Income < EUR 877.90 /month for no contribution	Co-payment based on the user's monthly income, with thresholds determining the percentage of contribution required: EUR 877.90 - 3 233.10 per month: range 0% to 90% of the assistance plan amount. EUR 3 233.10 per month or more: 90% of the assistance plan amount.	-
	Allocation Personnalisée d'Autonomie (APA) for residential care	In-kind	N/A	Contribution varies based on user's income and GIR level. EUR 2 676.09 /month or less: user pays GIR 5-6 rate. EUR 2 676.09 – 4 117.06 /month: user pays GIR 5-6 rate plus 0% to 80% of the difference between the user's GIR rate and the GIR 5-6 rate. EUR 4 117.06 /month or more: user pays GIR 5-6 rate plus 80% of the difference between the user's GIR rate and the GIR 5-6 rate.	-
	Aide sociale à l'hébergement (ASH)	Both	N/A	Users pay up to 90% of their income for accommodation costs. The county council pays the portion not covered by the user's income directly to the establishment. However, in the context of family accommodation, it is paid to the beneficiary or their guardian.	10% of user's income or EUR 121 per month
Germany	Pflegesachleistungen Institutional care	In-kind	N/A	The insurance fund provides fixed amounts to pay for the care costs, which is based on the care level of the user. For institutional care, the length of stay in the facility is also considered to estimate the cost. If the total expenses exceed the covered amount, the user pays the difference as participation.	-
Ireland	Home support services	In-kind	N/A	No cost sharing. Service is free of charge.	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
	Nursing home support scheme	In-kind	N/A	Users pay up to 80% of their assessable income and 7.5% of the value of any assets (in excess of an asset disregard) per annum. Any rental income received from their home (principal residence) is exempt from the assessment. The first EUR 36 000.00 of their assets is also exempt from the assessment. Principal residence will only be included in the financial assessment for the first 3 years of living in a nursing home. Meaning users will not pay more than 22.5% (7.5% a year, for 3 years) of the value of their home. Rules vary for beneficiaries with partners.	20% user's income or 20% of the maximum rate of the State Pension (non-contributory), whichever is the greater.
Israel	Home care via the National Insurance Institute	Both	Income < NIS 12 536	Single: - Up to NIS 12,536 - Full benefit - Over NIS 12,536 and up to NIS 18,804 - Benefit reduced by 50% - Over NIS 18,804 - No entitlement to benefit Couple: - Up to NIS 18,804 - Full benefit - Over NIS 18,804 and up to NIS 28,206 - Benefit reduced by 50% - Over NIS 28,206 - No entitlement to benefit	-
Italy (South Tyrol)	Home care services (Assistenza domiciliare)	In-kind	N/A	Co-payment based on the 'value of economic situation (VSE), which is the economic situation divided by the needs of the family unit. If the value of the economic situation is equal to 1, then it means that the economic situation covers exactly the needs of the household. Up to VSE is equal to 1.1 payment of the minimum fee is required (EUR 3.9 per hour). The participation increases then linearly as VSE increases, starting from the minimum fee until reaching the maximum fee. If the VSE is higher than 4.5, payment of the maximum fee is required (EUR 24 per hour).	-
	Residential services	In-kind	N/A	Co-payment based on income and assets of the user and their spouse/partner. In the event that their income and assets are not sufficient to cover the entire fee, the children are responsible for supplementing the remaining part. Any residual amount is covered by the municipality of residence.	Min guaranteed economic condition ³ 50% of economic situation

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
Japan	Long-term Insurance System (介護保険制度)	In-kind	10% co-payment rate if: < JPY 1.6 million per year 20% co-payment rate if: JPY 1.6 – 2.8 million per year 30% co-payment rate if: > JPY 2.8 million per year These values are for single use. Thresholds vary for those who have a household.	Operates through an insurance model where policyholders pay premiums and co-payments for services. The LTC fund, partially subsidised by various levels of government, reimburses service providers. Funding sources include premiums from those aged 40-64 and 65+, local municipal budgets, prefectural budgets, and the national government. Co-payment rates range from 10% to 30% based on income, with higher earners paying more. Income and asset tests determine maximum out-of-pocket limits and co-payment ceilings, particularly for nursing home residents. These limits vary based on income levels, with specific allowances for high-cost care and nursing home expenses.	-
Korea	Long-Term Care Insurance for the Elderly (노인장기요양보험제도)	Both	Standard Median Income (SMI) sets the threshold and varies by household structure. Out-of-pocket costs will be zero if: 1) Income is less than 30% of SMI (i.e. eligibility for social assistance); 2) Income is less than 40% of SMI (i.e. eligibility for medical assistance); or 3) Income is less than 50% of SMI and not on social assistance due to means-testing (commonly called “Next Level Tier”)	To qualify for a reduction in co-payments, beneficiaries must apply to the National Health Insurance Fund (NHIF), which reviews eligibility based on the beneficiary's health insurance premium record. Those eligible are classified as follows: a 100% reduction applies to individuals on social assistance (with limited assets, dependents, and income below 50% of the national median) or those with income below 50% of the national median without social assistance eligibility; a 60% reduction is given to those whose insurance premiums fall in the lowest 0-25th percentile; and a 40% reduction is for those with premiums in the 25-50th percentile range.	-
Lithuania	Social care	In-kind	N/A	Up to 20% of user's income, if they live alone or in a family where each member's income is up to three times the state-supported income level. If it exceeds three times the state-supported income level, payment for social care is up to 50% of user's income.	-
	Social assistance Institutional care	In-kind	N/A	Up to 80% of user's income if their property values is lower than the property value norm established in the municipality of their place of residence. However, if their property value exceeds this norm, the payment increases by 1% per month for each unit of property value that surpasses the established norm.	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
Luxembourg	Long-term care insurance	In-kind	N/A	No cost-sharing. For institutional care, the cost of board and lodging (which includes the cost of renting a room, meals and supervision) is always borne by the user. If the user is unable to pay, the 'complément accueil gérontologique' supplement can be requested to help with the costs.	-
	Complément accueil gérontologique	In-kind	Movable assets < EUR 23 610.75	The supplement is calculated using the beneficiary's income and assets, the cost of the accommodation and a deduction for personal needs. The accommodation cost is capped by law, with the maximum price depending on the institution's quality points and room features, ranging from EUR 3,200.35 per month for a private room to EUR 2,884.15 per month for a shared room, adjusted for any missing quality features.	EUR 538.33 per month
Netherlands	Social Support Act 2015 (Wet Maatschappelijke Ondersteuning 2015, Wmo)	In-kind	Varies by municipality	Varies according to municipality, the co-payment for long-term care services involves a subscription rate for both personalised and general services with ongoing assistance. Starting January 1, 2024, the maximum rate is EUR 20.60 per month, but municipalities can set lower rates or waive fees for low-income households or certain facilities.	-
	Long-Term Care Act (Wet Langdurige Zorg, WLZ)	In-kind	N/A	There are two rates that users pay depending on the outcome of means-test calculation: Low contribution: min EUR 200.40, max EUR 1 052.20 per month High contribution: max EUR 2 887.40 per month	-
New Zealand	Home support services	In-kind	For Community Services Card, gross income should be: < NZD 35 213.00 per year (single, living alone) < NZD 37 531 per year (NZ Super pensioner, living alone) Thresholds vary depending on family composition and if receiving NZ Super pension	The user can choose to have a whānau or family member, or an external carer to provide their care. The carer can then be employed by a Home and Community Support Service provider to deliver the agreed services. Personal care (ADLs) are funded by the Health New Zealand. Those who require domestic assistance (housework) need a Community Services Card to help with the costs.	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
	Residential care	In-kind	If user is aged > 65, assets < NZD 284 636.00 to qualify for government funding	A means test determines what the user must contribute to the cost of their care. The level of subsidy will depend on the type of care the person is assessed as requiring. A weekly personal allowance and an annual clothing allowance are paid separately to the person.	* NZD 55.35 per week as personal allowance, and * NZD 347.17 per year as clothing allowance.
Poland	Home care services	In-kind	People whose income is equal to, or lower than, the minimum level of income set are exempted from payments (i.e. PLN 776.00 per month for single person and PLN 600.00 gross for household).	The amount payable depends on the average income per person in the family. Partial or total exemption from such payments is possible in particularly justified cases.	-
	Residential care	In-kind	N/A	Users pay up to 70% of their income. If the user makes a payment which is lower than the maintenance costs, the family is then obliged to pay the remaining amount; though only where the income per person in the family exceeds 300% of the income criterion set (PLN 2 328.00 for single person, PLN 1 800.00 for household) and, provided that the amount which remains after making the payment is at least 300% of this level. The municipality concerned pays the difference. If the person has no family, or if the family is exempted from payment because of their low income, the municipality covers the whole amount due.	-
Portugal	Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	In-kind	N/A	Monthly contribution is calculated as a percentage of the family's income, ranging from 75% to 90%. This percentage is adjusted based on the elderly individual's level of dependence.	-
Slovenia	Home help [pomoč na domu]	In-kind	N/A	Service hours: 20 hours per week, or 25 hours per week in exceptional cases.	1.5 times the Slovenian minimum income
Spain	Ayuda al domicilio	In-kind	Varies by Autonomous Communities but usually exemption for beneficiaries earning less than IPREM (EUR 600.00 per month in 2024)	Up to each municipality Co-payment is based on an individual's economic capacity, which includes income plus 5% of assets (excluding the primary residence), relative to the IPREM. Individuals pay a minimum of EUR 20.00 per hour, while those with income below the IPREM receive full cost coverage. The maximum out-of-pocket contribution is capped at 90% of the service cost. Regulations vary by autonomous community, as each sets its own guidelines for assessing economic capacity and determining co-payment amounts.	-

Countries and subnational areas	Benefit and schemes	Benefit type	Threshold below which a person receives the full benefit	Co-payment/benefit mechanism	Min. reserve after cost and benefit receipt
	Atención Residencial	In-kind		Contributions first cover accommodation and personal care expenses according to the individual's economic capacity. The economic participation is calculated using a formula that ensures progressive contributions, where the beneficiary's participation (PB) is determined by subtracting the minimum personal expenses from their economic capacity. If this calculation results in a negative number, the individual is not required to contribute.	19% of the monthly IPREM
Sweden	Home care (hemtjänst)	In-kind	N/A	Only users receiving more than 6 hours of care per month are required to pay a co-payment. Users pay up to SEK 2 575.00 per month for care depending on the amount of hour and type of services required.	SEK 7,062 per month for single people
	Institutional care (särskilt boende)	In-kind	N/A	Users pay up to SEK 2 575.00 per month for care. Food fee is covered 100% by the user and is capped (may vary depending on municipality). There is housing allowance available to help pay the rent of the room.	SEK 382 per month
United Kingdom	Social care	Both	N/A	Depending on income levels: < GBP 14 250.00: user pays what they can afford from income. GBP 14 250.00 – 23 250.00: user pays what they can afford from income plus a contribution from assets (GBP 1.00 per week for every GBP 250.00 of assets between the limits). > GBP 23 250.00: full cost covered out-of-pocket.	GBP 30.15 per week.
United States (California)	In-Home Supportive Services (IHSS)	In-kind	Income < 138% of the Federal Poverty Level (FPL). For 2024, this is approximately USD 1 680.00 per month for an individual.	Service hours: 195 hours per month for those who do not have severe impairments, while users severely limited in their functional ability can receive up to approximately 283 hours per month.	USD 600.00 per month that varies depending on family size
	Medi-Cal Institutional Care	In-kind	N/A	No income limit, but most of the user's income is used to pay nursing home fee.	USD 35.00 per month personal expenses plus any Medicare or insurance premiums (medical, dental, etc.)

Note: 1 Providers can ask clients to pay this fee, but not all of them do. 2 The amount paid as income-tested care fee in Home Care Package benefit also counts toward this cap. 3 Guaranteed economic condition is the portion of the economic situation not considered for the purpose of calculating co-payment fee. Min reserved amount: Lists the minimum amount of the benefit provided, or the specifics of care services offered.

Source: OECD analyses based on Long-Term Care Social Protection questionnaire.

11. In addition to means-testing, LTC benefits in most countries are determined by the recipient's level of care needs, shown in Table 9. Table 4 below outlines the minimum government contribution per benefit in each country and the criteria used to determine the benefit value. While some countries allocate benefits strictly according to the level of care required, others also take the recipient's financial situation into account. In countries such as Australia and France, a more complex design is used, where part of the benefit is paid as a flat fee, another part is adjusted based on care needs, and additional adjustments are made according to the individual's income.

12. In contrast, countries like Austria, Belgium, and Poland opt for a simpler approach for certain benefits by offering a fixed amount. For instance, Austria's 24-hour care allowance, Belgium's incontinence allowance and care budget for severe care needs, and Poland's care supplement and *Zasitek pielęgnacyjny* all provide fixed amounts that do not vary based on the level of care required, even though eligibility is still determined by care needs. This is often the case when a supplementary benefit is necessary in addition to the main benefit. As a result, while the main LTC benefits generally depend on the level of care needed, supplementary benefits may be provided as fixed amounts.

Table 4. Government minimum contribution in LTC benefits.

Countries	Benefit and schemes	Type	Minimum public support	Relevant criteria to calculate benefit amounts
Australia	Home Care Package	In-kind	Government contribution daily rate: Level 1: AUD 29.01 If client is eligible to pay an income tested care fee, the government subsidy is reduced by this amount.	Government subsidy is based on individual care needs, and supplements for specific care need is added if the user is eligible.
	Aged care home	In-kind	Government contribution is a flat daily rate based on different care profiles, as examples: - Independent without compounding factors: AUD 48.23 - Assisted mobility, low cognition: AUD 137.06 - Not mobile, lower function, higher pressure sore risk, with compounding factors: AUD 253.82	
Austria (Viena)	Care allowance (Pflegegeld)	Cash	For category 1, EUR 192.00 per month	Flat monthly rate based on the 7 care categories (the amount of care hours).
	24-hour care	Cash	EUR 400.00 per month per carer for a self-employed	Flat fee based on the number of carers (max 2 per month) and type of carer contract.
	Fonds Soziales Wien	In-kind	Not specified	Based on care needs, and user's income.
Belgium (Flanders)	Care budget for people with severe care needs	Cash	EUR 140.00 per month	Flat rate.
	Care budget for the elderly with care needs	Cash	For category 1, EUR 106.00 per month	Flat monthly rate based on the 5 care categories (based on care needs and family income).
	Allowance for the chronically ill	Cash	First criteria group, EUR 371.55 per calendar year	Flat yearly rate based on 3 groups, which encompass different sets of criteria.
	Incontinence allowance	Cash	EUR 610.53 per year	Flat fee.

Countries	Benefit and schemes	Type	Minimum public support	Relevant criteria to calculate benefit amounts
	Home care organisations	In-kind	Not specified	Based on care needs and user's income.
Canada (Ontario)	Home and Community Care	In-kind	Not specified	Based on number of care hours per month.
	Long-term Care Homes	In-kind	Not specified	Government subsidy for personal and nursing care based on needs.
Czechia	Care allowance (Příspěvek na péči)	Cash	Level I: CZK 880	Flat monthly rate based on the 4 care categories (unmanaged needs).
Denmark	Home help (Hjemmehjælp)	In-kind	100% covered	Only based on care needs.
	Nursing Home Housing (Plejhjemsboliger)	In-kind	Care component covered 100%	
Estonia (Tallinn)	Domestic care service (koduteenus)	In-kind	Not specified	Based on user's income.
	24/7 general care (Ööpäevane üldhooldus)	In-kind	Care component EUR 734.00	Care component is based on needs assessment. However, the government may contribute more depending on user's income and assets.
Finland	Care allowance (Eläköntä saavan hoitotuki)	Cash	Basic rate EUR 83.34 per month	Rates based on care needs
	Home care services (Kotihoito and Tukipalvelut) Institutional care	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
France	Allocation Personnalisée d'Autonomie (APA)	Cash	Not minimum set, but maximum amount for GIR 4 is EUR 762.87	User's needs and means considered in determining the benefit amount.
	Aide sociale à l'hébergement (ASH)	Both	Not specified	Government contributes the remaining amount not covered by the user's income and assets.
Germany	Pflegegeld (Care Allowance)	Cash	No minimum set, but maximum amount for care level 2 is EUR 332 per month	Based on care needs.
	Pflegesachleistungen (Care Benefits In-Kind)	In-kind	Care level 2 is EUR 761 per month	Based on care needs.
	Institutional care	In-kind	Care level 2 is EUR 770 per month	Based on care needs.
Ireland	Home support service	In-kind	100% covered.	Based on care needs.
	Nursing home support scheme	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Israel	Home care via the National Insurance Institute	Both	Not specified	Based in income and needs.
Italy (South Tyrol)	Care allowance (Assegno di cura)	Both	Level 1: EUR 576.5	Flat rate based on the 4 care levels.
	Home care services (Assistenza domiciliare) Residential services	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income and assets.
Japan	Long-term Insurance System (介護保険制度)	In-kind	- 12.5% is funded by local municipal budgets; - 12.5% is funded by prefectural budgets (or 17.5% for LTC facilities); - 25% is funded by the national government (or 20% for LTC facilities).	Policyholders pay insurance premiums and if they are to receive LTC services, they also pay service fees to care providers (i.e. co-payment). The service providers then claim reimbursement from the special LTC insurance fund or budget.
Korea	Long-Term Care Insurance for the Elderly	Both	Cognitive support: KRW 643 700.00	Flat monthly rate based on the 6 care categories (based on cognition)

Countries	Benefit and schemes	Type	Minimum public support	Relevant criteria to calculate benefit amounts
	(노인장기요양보험제도)			status and needs).
Lithuania	Social care Social assistance Institutional care	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Luxembourg	Long-term care insurance	Both	Costs of the care and assistance required for the user to perform ADLs are 100% financed. The min amount of assistance provided is Level 1, from 210 to 350 minutes of care per week.	15 levels based on the amount of care and assistance required.
	Complément accueil gérontologique	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Netherlands	Social Support Act 2015 (Wet Maatschappelijke Ondersteuning 2015, Wmo)	Both	Not specified	Users pay a monthly rate of EUR 20.60, then government pays the remaining.
	Long-Term Care Act (Wet Langdurige Zorg, WLZ)	Both	EUR 200.40 per month	Based on user's income and assets.
New Zealand	Home support services Residential care	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Poland	Zasilek pielęgnacyjny	Cash	PLN 215.84 per month	Flat fee.
	Care supplement (dodatek pielęgnacyjny)	Cash	PLN 330.07 per month	Flat fee.
	Home care services Residential care	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Portugal	Complement by dependency (for home care)	Cash	1st degree, EUR 122 per month	The amount corresponds to a percentage of the value of the Social Pension and vary according to the degree of dependency.
	Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
Slovenia	Assistance and Attendance Allowance (dodatek za pomoč in postrežbo)	Cash	Lower rate EUR 180.85	Flat monthly rate based on the 3 care levels.
	Home help [pomoč na domu]	In-kind	50% of cost financed	Based on user's income.
Spain	Ayuda al domicilio	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.
	Prestación económica vinculada al servicio Prestación económica de asistencia personal	Cash	Grado I: EUR 100 per month	Flat fee that varies by each care level.
	Atención Residencial	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income and assets.
Sweden	Home care (hemtjänst) Institutional care (särskilt boende)	In-kind	Not specified	Government contributes the remaining amount not covered by the user's max payment rate SEK 2 575 per month.
United Kingdom	Attendance allowance	Cash	Low-rate: GBP 72.65	Flat fee that varies depending on care needs.

Countries	Benefit and schemes	Type	Minimum public support	Relevant criteria to calculate benefit amounts
	Social care	Both	Not specified	Government contributes the remaining amount not covered by the user's income and assets.
United States (California)	In-Home Supportive Services (IHSS) Medi-Cal Institutional Care	In-kind	Not specified	Government contributes the remaining amount not covered by the user's income.

Note: Min government contribution: The minimum financial contribution or support provided by the government for each benefit or scheme. Details on benefit value based on: The criteria or basis on which the benefit value is determined. It might include factors like individual care needs, care hours, or specific care categories.

Source: OECD analyses based on Long-Term Care Social Protection questionnaire.

13. As described in Table 3, many of the countries that apply income testing to decide on a person's eligibility to LTC benefits define an income threshold below which the individual with care needs receives the full benefit available for the respective severity of needs. Table 5 compares these thresholds with the national poverty lines: the relative poverty threshold at 50% of the median income, and the threshold where people are considered "at risk of poverty" which equals to 60% of the national median income. In most of the countries shown below the threshold is set below the poverty line. In Vienna (Austria), for example, that threshold is set at EUR 626, roughly half of the relative poverty threshold. This raises concerns about the poverty risks associated with the dependence on formal LTC. The last column shows how much of the LTC costs is covered by public support for an older person with moderate LTC needs, receiving care at home and earning an income equal to the poverty threshold. While most countries cover between 66 and 100% of the cost (except for Croatia), the remaining private copayments may consume large parts of these people's incomes. This is especially problematic when people receive care at home and need to cover other expenses such as housing, food, clothing, transport etc.

Table 5. Income-testing thresholds for long-term care benefits

Countries and subnational areas	Benefits and schemes	Threshold below which a person receives the full benefit	Relative poverty threshold (50%)	AROP threshold (60%)	Public support
Vienna (Austria)	Fonds Soziales Wien	EUR 626	EUR 1 222.64	EUR 1 467.17	94%
Flanders (Belgium)	Care budget for the elderly with care needs	EUR 1 437.38	EUR 1 184.85	EUR 1 421.82	100%
Croatia	Subsidised home care for people with a low income	HRK 1 750	HRK 2 907.92	HRK 3 489.50	8%
Tallinn (Estonia)	Domestic care service (koduteenus)	EUR 360/month	EUR 576.22	EUR 691.47	100%
France	Allocation Personnalisée d'Autonomie (APA) for home help	EUR 877.90/month	EUR 993.33	EUR 1 192.00	75%
Reykjavik (Iceland)	Social home service	ISK 52 380.08	ISK 225 206.46	ISK 270 247.76	100%
Japan	Support for institutional care costs Supplementary support for institutional care	JPY 133 333.33	JPY 103 333.33	JPY 124 000	86%
Korea	Long-Term Care Insurance for the Elderly	KRW 501 631.50	KRW 1 249 166.66	KRW 1 499 000	85%
Latvia	Home care services / care allowance	EUR 128.06	EUR 442.65	EUR 531.18	74%
Spain	Ayuda al domicilio	EUR 600	EUR 753.74	EUR 904.49	82%
Illinois (United States)	Home and Community Based Services	USD 1 680.00	USD 1 942.71	USD 2 331.25	66%

Note: The relative poverty income is 50% of the population-wide median income. The column public support represents the share of total costs that are covered by the public social protection system for an older person earning 50% of the median income with moderate needs receiving care at home (see Box 1), taking into account all LTC benefits and schemes available in each country and region.

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Wealth-testing must be well-designed to avoid distorting saving behaviour

14. Wealth-testing can help target limited public social protection budgets to older people who need it most. Countries that apply asset-testing cover a lower share of the total costs of care for older people with higher wealth. This is done under the assumption that, even if they cannot afford out-of-pocket costs from their incomes alone, people with greater wealth can afford to cover the shortfall from their assets. Consequently, older people in these countries may have to deplete their assets to reach a threshold that qualifies them for increased public support. Across countries, asset-testing is used to determine the level of support or cost-sharing, not eligibility to the benefit. There are sometimes exceptions to this rule. In Croatia, the right to assistance and care allowance cannot be granted to a person who has a life or life support contract, who owns an apartment or house other than the apartment or house that he or she uses as a dwelling, which can alienate or rent and thereby provide assistance and care or who owns a business premises that he does not use to carry on his registered business.

15. Most OECD countries have asset-tested LTC care benefits (see Table 6). Across the different benefits and schemes, there are a set of common features that asset-testing tends to have. First, assets below a certain value are often excluded from asset-testing. However, these thresholds tend to be low compared to the national mean net wealth, and if public support is not effective below these asset thresholds, then care recipients may still end up using their assets to pay for care. Second, often only a share of all assets above the threshold are considered for asset-testing (although some countries do consider all assets). Third, asset-testing may include or exclude certain types of assets. For instance, the care recipient's primary residence is often excluded from asset-testing when the older person or their dependents are living there, like in France or Slovenia for home care, or intend to keep living there after the care recipient's death, like in Germany. Fourth, deferred payment agreements may be used to allow care recipients to postpone user contributions to their care. In such schemes, care recipients agree to use their assets (including their primary residence) to repay the public social protection system for any postponed user contributions. This happens if they sell their house (e.g. when moving to institutional care) or when they die.

16. Several countries apply asset testing also on the primary residence. In most cases, the primary residence is considered if no dependent lives in and if it is above a specific monetary value. Flanders, Belgium, and Spain define a specific percentage of all assets, while the Netherlands define a specific percentage of assets from a certain threshold. An assessment of the distribution of assets and its median values could be used to determine such thresholds or percentage, using either tax data or a national household survey.

17. There is a risk that asset-testing can act as a form of taxation on wealth and savings, thus having the potential to introduce distortions in saving behaviours. Given a prevalent myopic behaviour in saving for LTC, the distortion effect may be small but is difficult to quantify it. Introducing wealth-testing with low thresholds may lead to unintended consequences, such as strategic behaviour by older individuals to fall below the threshold. To address this, countries have implemented preventive measures. For instance, in the United States, in 2016, the Deficit Reduction Act of 2005 was signed into law, extending the look-back period on wealth transfers to five years and shifting the penalty period from the time of improper transfer to the time of Medicaid application. This measure resulted in an 11% reduction in transfers to children and a decrease of \$4 860 in the average total amount of transfers before application (Liu and Mukherjee, 2020^[1]). Similarly, New Zealand has implemented a five-year look-back period for wealth flows, with exceptions for gifting wealth in recognition of care (Ministry of Social Development, 2024^[2]). An alternative

approach is to make an assumption on capital gains based on a given amount of wealth, as is already done in Flanders (Belgium) and the Netherlands, so that the distortionary effect of setting a certain threshold can be avoided. These approaches help to mitigate the risk of strategic behaviour and ensure that wealth-testing effectively targets those most in need of LTC support.

Table 6. Treatment of assets in LTC benefits

Countries and subnational areas	Benefits and schemes	Setting	Simplified description of rule	Types of assets	Deferred payment
Flanders (Belgium)	Allowance for the assistance of the elderly	Both	6% of assets	Primary residence excluded	No
Croatia	Allowance for assistance and care	Institution Home	None Not given out if care recipient has assets	All All	Yes No
England	Social care	Institution Home	No user contribution for assets below GBP 14 250, full contribution for assets above GBP 23 250	All Primary residence excluded	Yes Yes
Estonia	Institutional care	Institution	Full user contribution if care recipient has assets	All	No
France	Allocation Personnalisée d'Autonomie Aide sociale à l'hébergement	Both Institution	100% of assets can be used for contributions None	Primary residence excluded All	No Yes
Germany	Assistance for care (Hilfe zur Pflege)	Both	EUR 10 000 in liquid assets per person, an adequate car, additional state-subsidised retirement funds, and primary residence excluded	All	No
Hungary	Homes for the elderly	Institution	Full user contribution if care recipient has assets; higher income allowance for care recipients that have assets	All	No
Japan	Long-term care insurance	Institution	To qualify for reduced board and lodging fees care recipients must have assets worth below JPY 10 000 000	Excludes life insurance, cars, watches	No
Lithuania	Institutional care	Institution	1% of assets over EUR 4 260*	All	No
Luxembourg	Complément accueil gérontologique	Institution	None	All	Yes
Netherlands	Wet langdurige zorg (Wlz) Wet Maatschappelijke Ondersteuning (Wmo)	Both	4% of assets over EUR 31 747 for singles or EUR 63 494 for applicants with partner	All	No
Slovenia	Municipality-subsidised care	Institution Home	100% of assets over EUR 2 500	All Primary residence excluded	No
Spain	Ayuda al domicilio Prestación económica vinculada al servicio Atención Residencial	Home Institution	5% of assets None None	Primary residence excluded All All	No No No
Illinois (United States)	Home and Community Based Services Medicaid institutional care	Home Institution	Full user contribution if care recipient has assets worth over USD 2 000	Primary residence excluded Primary residence excluded only if	No No

				worth less than USD 585 000	
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Note: Countries and subnational areas are sorted top to bottom alphabetically by the name of the country. * based on a value of EUR 355 per square meter for a property with 12 square meters.

Source: OECD analyses based on the OECD Long-Term Care Social Protection questionnaire

Eligibility for benefits towards informal carers aims to target those providing extensive care

18. Most OECD countries with a cash benefit to informal carers are European countries. Canada, Colombia, Japan and Korea do not have a cash benefit for informal carers. However, they have developed systems of care leave to help workers combine care and work. In most countries, the age requirement for caregivers is set at the legal age of 18. However, some countries allow individuals as young as 16 to serve as caregivers and receive benefits, as illustrated in Table 7.

19. All countries have to delineate the pool of beneficiaries of the cash benefits. In most countries, means-testing is the central entitlement condition to limit the pool to people on low-incomes - this condition is commonly found in social assistance programmes. With means-testing, countries aim to strike the right balance between avoiding to trap carers in a low-paid informal care position and providing sufficient income assistance. Means-testing delineates the potential beneficiaries to those on poor incomes – one of the main goals of social protection is to protect against poverty. In general, means-testing is related to the carer's household. When the care recipient has to be a co-resident (e.g. Spain, Portugal), means-testing applies to both the carer and the care recipient.

20. While some countries do not specify a minimum required amount of care, using broad terms such as "full-time care," others delineate specific time commitments that caregivers are expected to meet. Cohabitation requirements appear to be relatively relaxed across many regions. In countries like Spain, Portugal, and Ireland, caregivers are explicitly required to reside with the person they are caring for. In contrast, other countries do not mandate cohabitation but emphasise the importance of caregivers being readily available, at hand, or able to respond promptly to the care recipient's needs. This expectation is often linked to the fact that some benefits necessitate caregivers undergoing a needs assessment, which is frequently conducted alongside the assessment for LTC benefits for the care recipient. For instance, in Finland and Luxembourg, the needs assessment for caregivers evaluates their level of commitment and the complexities associated with their family member's care needs. Based on the results of these assessments, additional services to support informal care are then tailored to meet the requirements of both the caregiver and the care recipient.

21. Luxembourg, New Zealand, and Sweden (anhöriganställning, not available in all municipalities) offer caregiver benefits that differ significantly from traditional allowances. In these regions, municipalities may employ family members directly to provide care, rather than employing external care providers. This approach supports a more personalised care experience, as family caregivers typically have an intimate understanding of the care recipient's needs, preferences, and routines. Additionally, employing family members for caregiving can alleviate the burden on formal care systems.

22. Several countries (e.g. France, Germany, and the Netherlands) require that care recipients make a formal contract (or health plan) with their caring relatives to formalise the arrangement. In Germany, carers can be affiliated to the LTC fund if the care recipient receives an allowance. Once registered, the LTC fund pays for the social security contributions of the carers and provides free training, but does not establish a contract of care with a formal transfer of money. There is, however, the possibility for the carer to formally receive part of the cash benefit of the care recipient to compensate for care. In the United States, some public programmes (Medicaid, some state-revenue only programs, Veterans-Directed Care)

enable older people eligible for home care to hire family members. A compensation is paid to family caregivers as an hourly wage and is subject to the Fair Labor Standards Act minimum wage and overtime pay requirements.

Table 7. Eligibility for informal caregiver cash benefits

Countries and subnational areas	Benefit and schemes	Age	Min amount of care provided	Residency	Cohabitation requirement	Additional criteria
Finland	Informal care benefit (Oma. ishoidon tuki)	> 18	Not specified	Not specified	No ¹	The level of commitment and difficulty of the family member's care is assessed
Ireland	Carer's allowance	> 18	> 35 hours per week, over 5 to 7 days	All residents	No	* The person cared for is over the age of 16 and likely to need care for at least 12 months. * Not be employed, self-employed, doing voluntary work, training, or any education courses for > 18.5 hours per week. * Not living in a hospital, convalescent home or similar institution.
	Carer's support grant ²	> 16	Full-time	All residents	No ³	* Caring for the person for at least 6 months and this period includes the first Thursday in June. * Not be employed, self-employed, doing voluntary work, training, or any education courses for > 18.5 hours per week. * Not recipient of Jobseeker's Allowance or Jobseeker's Benefit * Not signed on for credited contributions. * Not living in a hospital, convalescent home or similar institution.
	Carer's benefit	> 16	Full-time care	All residents	No	* Contribution to social insurance (PRSI) * Employed for at least 8 weeks in the previous 26-week for a minimum of 16 hours per week or 32 hours a fortnight⁴. * Not be employed, self-employed, doing voluntary work, training, or any education courses for > 18.5 hours per week. * Income of < EUR 450.00 per week after tax. * Not living in a hospital, convalescent home or similar institution.
Luxembourg	Long-term care insurance (caregiver payment)	Not specified	At least once a week	Not specified	Not specified	* The caregiver must be registered and assessed by the State Office for Assessment and Monitoring of the long-term care insurance (AEC).
New Zealand	Home support services (family member)	> 16	Not specified	All residents	Not specified	* The person cared for must receive Home Support Services benefit.
	Supported living payment	> 18 ⁵	Full-time care	Resident for > 2 consecutive years	No	* Provide care at home to someone other than a spouse or partner, and that person must otherwise require hospital or residential-level care. * Income-tested.

Countries and subnational areas	Benefit and schemes	Age	Min amount of care provided	Residency	Cohabitation requirement	Additional criteria
Portugal	Support allowance for informal carers (Subsídio de apoio ao cuidador informal principal)	18 - 66	Not specified	All residents	Yes	* The reference income of the main informal caregiver's household must be below EUR 662.04 per month (1.3 times the Social Support Index – IAS). * Relationship with the person being cared for: Spouse or de facto partner, relative or in-law up to the 4th degree of the direct line or collateral line. * The person cared for is awarded one of the dependency benefits. * Not receiving remuneration for professional activity or unemployment benefits.
Spain	Prestación económica por cuidados en el entorno familiar	> 18	Not specified	All residents	Yes ⁶	* Relationship with the person being cared for: Spouse or de facto partner, relative or in-law up to the 4th degree of kinship. * Care provided for at least one year prior to the date of application submission.
Sweden (Stochhold Stad)	Attendance allowance	Not specified	Not specified	Not specified	No	* Relationship with the person being cared for: family member, relative, good friend or neighbour.
United Kingdom (England)	Carer's allowance	> 16	> 35 hours a week	Resident for at least 2 of the last 3 years.	No	* The person cared for must get certain benefits. * Not in full-time education or studying for > 21 hours per week. * Income of < GBP 151 per week after tax, National Insurance and expenses.

1. Not a requirement, but those who live in the same neighbourhood, nearby, get support more easily than those living far away.
2. This benefit is given automatically to beneficiaries of Carer's Allowance (both full rate and half-rate), Carer's Benefit or Domiciliary Care Allowance.
3. Certain criteria must be met: the person must be able to contact you quickly and directly.
4. This criterion does not apply if the applicant received Carer's Benefit in the previous 26 weeks.
5. 18 years or older and do not have a dependent child, or 20 years or older and have a dependent child.
6. When the person being cared for has dependency grade II or III. Those with grade I dependency are exempt of this criterion.

23. Table 8 provides examples of how informal caregiver cash benefits can depend on the factors discussed above. In Finland, for example, informal care benefits vary by level of care required, offering monthly rates from EUR 461.99 to EUR 1 608.61 depending on the severity of a person's needs. Ireland's carer benefits include a means-tested carer's allowance (up to EUR 286 per week for a carer above the age of 66), an annual carer's support grant of EUR 1 850, and a carer's benefit capped at EUR 373.50 per week for multiple dependents. Luxembourg's LTC insurance is categorised into ten levels, paying from EUR 12.50 to EUR 262.50 per week.

Table 8. Informal caregiver cash benefit mechanism

Countries and subnational areas	Benefit and schemes	Payment rules	Amount
Finland From Western UUsimaa	Informal care benefit (Omaishoidon tuki)	Benefit depends on the required level of commitment and difficulty of care duties. 3 categories based on care needs: 1. Daily recurring need for care 2. Around -the-clock need for care 3. Very demanding stages	From Western UUsimaa 1. EUR 461.99 per month 2. EUR 923.99 per month 3. EUR 1 608.61 per month
Ireland	Carer's allowance	Means-tested assessment conducted to calculate benefit amount, with a sliding scale that varies by age. Combination with other benefit may result in half-rate payments.	Max full rate: Carer age < 66 EUR 248.00 per week Carer age > 66 EUR 286.00 per week
	Carer's support grant	Annual flat rate.	EUR 1 850.00 per year
	Carer's benefit	Weekly flat rate that varies based on the number of people being cared for. It is only provided for 2 years (104 weeks).	Caring for 1 person EUR 249.00 per week Caring for more than 1 person EUR 373.50 per week
Luxembourg	Long-term care insurance for caregiver	10 different categories which vary depending on the amount of assistance provided by the caregiver.	Category 1. EUR 12.50 per week Category 10. EUR 262.50 per week
New Zealand	Home support services (family member)	Caregiver is employed by the municipality	Rates not available
	Supported living payment	Benefit depends on the applicant's situation, and their income. Income threshold - Adult (18+) single < NZD 787.00 per week, or < NZD 40 920.00 per year Income threshold - Married (with children) < NZD 1 238.00 per week, or < NZD 64 342.00 per year	Adult (18+) single NZD 402.84 per week Married (with children) NZD 718.14 per week
Portugal	Support allowance for informal carers (Subsidio de apoio ao cuidador informal principal)	Benefit amount is equal to the difference between the sum of the income of the main informal caregiver and the reference value of the allowance (IAS for 2024, EUR 509.26)	Depending on income Max income EUR 662.04 per month, therefore max benefit EUR 407.41 per month

Countries and subnational areas	Benefit and schemes	Payment rules	Amount
Spain	Prestación económica por cuidados en el entorno familiar	The benefit amount is determined by each municipality and depends on the beneficiary's degree of dependency and economic capacity. While the law sets minimum and maximum benefit thresholds, municipalities apply their own methodology.	Min per month Grade I – EUR 100.00 Grade II – EUR 150.00 Grade III – EUR 200.00 Max per month Grade I – EUR 180.00 Grade II – EUR 315.00 Grade III – EUR 455.40
Sweden From Stockholm stad	Attendance allowance	The allowance can be distributed in twelve monthly instalments based on the percentage of care required. Group 1. 30% - Daily help, 1–2 times for care and supervision. Group 2. 60% - Help multiple times a day, at least three times for care and supervision. Group 3. 90% - Help multiple times a day, including overnight care. Group 4. 120% - Continuous care and supervision at all times.	Group 1: SEK 1 433.00 Group 2: SEK 2 865.00 Group 3: SEK 4 298.00 Group 4: SEK 5 730.00
United Kingdom	Carer's allowance	Weekly flat fee.	GBP 81.90 per week

2. Needs assessments for long-term care

24. Population ageing in the OECD is occurring at a rapid pace, primarily driven by advancements in life expectancy and a decline in fertility rates. As individuals age, their physical and mental health tends to decline, leading to potential challenges in performing routine tasks, including getting dressed, shopping, or even taking a walk. Consequently, older people will require a range of personal care and assistance services, commonly referred to as long-term care or LTC. This report uses the definition of LTC as described in the council recommendation on access to affordable high-quality LTC (European Commission, 2022^[3]). It is understood as “a range of services and assistance for people who, as a result of mental and/or physical frailty and/or disability over an extended period of time, depend on help with daily living activities and/or need some permanent nursing care. The daily living activities for which help is needed may be the self-care activities that a person must perform every day (Activities of Daily Living, ADL, such as bathing, dressing, eating, getting in and out of bed or a chair, moving around, using the toilet, and controlling bladder and bowel functions) or may be related to independent living (Instrumental Activities of Daily Living, IADL, such as preparing meals, managing money, shopping for groceries or personal items, performing light or heavy housework, and using a telephone).”

25. All countries and subnational areas have some form of LTC needs assessment to determine whether an older person would be entitled to public support. The exact procedures vary widely, with respect to who undertakes the assessment, which types of needs are considered, the scoring systems used to rate the severity of needs, whether social structures and external factors are taken into account, how scores are aggregated, and finally how scores relate to service and benefit levels.

26. First, countries differ with respect to the types of LTC needs that are considered in assessments. Across the OECD and the EU, countries use different constellations of ADLs, IADLs, social activities and

even, in some cases, factors like loneliness and self-assessed ability to cope with difficulties. This heterogeneity is reflected in the definitions used to determine eligibility, entitlement and level of support. Several countries have in the past decade amended their LTC needs assessments to include not only functional limitations, but also mental and cognitive impairments. Germany, for example, since 2015 recognises dementia as a condition that requires more support even if the person has no physical impairments.

27. Second, countries apply different scoring rules. Some assessments attribute different weights to different impairments and some require that specific needs be present for an older person to qualify for public support. In Germany, the highest weights are placed on 'self-sufficiency', a category that comprises 13 indicators, and within which items such as eating, drinking and using the toilet have higher weights than others. Mobility (comprised of five indicators) has the lowest weights. A number of countries set minimum thresholds for needs or vulnerability in order to become eligible for public support. Some countries and subnational areas have a defined number of categories based on the assessment scores. Some of the categories may not be eligible for any public support. The number of categories, how they aggregate scores, and how each type of LTC need contributes to the final score, all vary across countries, and sometimes regions and municipalities.

28. Third, not all countries have standardised LTC needs assessments. In Belgium, Canada and in the United States there are both federal and regional, state or municipal benefits and schemes, with their own specific rules and regulations. In Portugal, there is one needs assessment scale for cash benefits and another for in-kind care, this last one left at the discretion of the private care provider. Other countries (e.g. France, Germany, Netherlands and Spain) use a standardised assessment tool nationwide. In a number of Nordic countries, municipalities are responsible for undertaking the needs assessment and use their own criteria, although this has also changed over time, for instance, in Finland (see next section). Similarly, Estonia had also different needs assessment across municipalities and embarked on a process of harmonisation across the country and developed its own needs assessment tool instead of using an international tool.

29. Fourth, LTC needs assessments are conducted by different actors across countries and subnational areas. In Belgium and Finland, multidisciplinary teams conduct needs assessments. In Austria and England, assessments are based on a physician's evaluation. In Japan, a computer-aided initial assessment (based on an 85-item questionnaire) is used to assign each applicant to one of seven levels of LTC needs. The Care Needs Certification Board, a committee of medical and other professionals, then reviews the results.

Types of LTC needs considered in assessments

30. Countries consider different types of needs to determine a person's overall entitlement to public support or the level of the support provided. The types of needs are usually summarised under ADLs, IADLs, social activities, complemented by factors like loneliness and self-assessed ability to cope with difficulties (see Table 1) and every country chooses a constellation of these types. Almost all countries include some ADLs, IADLs and social activities in their needs assessments: Especially the ability to perform basic self-care tasks such as bathing, dressing, eating, toileting (ADLs) as well as activities necessary for independent living, such as managing finances, medication management, meal preparation (IADLs) is considered fundamental for a person to live at home by themselves. Only countries without an official or standardised needs assessment do not explicitly state that they assess ADLs and IADLs, such as Greece or Bulgaria.

31. While almost all countries take into account needs arising from ADLs, IADLs and cognitive functioning, some countries assess needs more comprehensively than others. For example, Austria, Japan, Korea, Slovenia and Spain predominantly look at ADLs, IADLs and cognitive state. Australia, Canada (Ontario), the Nordic countries, the United Kingdom (England) and the United States (California)

additionally consider social and emotional needs, safety and accessibility and family support in their needs assessment. In Portugal, IADLs are considered neither for home care nor for institutional care, with ADLs being the primary concern.

32. That being said, a growing number of countries are expanding their LTC needs assessments to explicitly consider social activities as well as cognitive abilities. Several countries have recently updated their systems to account for not only physical impairments but also mental and cognitive health. For example, Germany recognises dementia as a condition requiring significant support, even when physical limitations are absent. Similarly, Slovenia's Long-Term Care Act, adopted in 2023, introduced newly defined care categories and an updated assessment tool. This tool evaluates LTC needs based on eight criteria, including cognitive and communication abilities, behaviour, and mental health (Rupel, 2022^[4]). Australia is also in the midst of a major reform of its Aged Care Act, with the introduction of a streamlined, nationwide needs assessment system called the Integrated Assessment Tool (IAT) in July 2024. The IAT comprehensively assesses a wide range of factors, such as carer profiles, health, social and psychological needs, cognition, behaviour, home safety, and financial and legal aspects.

Table 9. Types of needs considered in the assessment

Countries and subnational areas	Benefit and schemes	ADL	IADL	Cognitive state	Social & emotional needs	Health conditions	Safety & accessibility	Family/carer support	Other
Australia	Commonwealth Home Support Programme Home Care Package Aged care home	●	●	●	●	●	●	●	●
Austria (Vienna)	Care allowance (Pflegegeld) 24-hour care Fonds Soziales Wien	●	●	●					
Canada (Ontario)	Home and Community Care	●	●	●	●	●		●	●
	Long-term Care Homes	●	●	●	●	●	●	●	●
Belgium (Flanders)	Care budget for people with severe care needs Care budget for the elderly with care needs Incontinence allowance Home care organisations	●	●	●	●	●	●	●	
Czechia	Care allowance (Příspěvek na péči)	●	●						
Denmark	Home help (Hjemmehjælp) Nursing Home Housing (Plejehjemsboliger)	●	●	●	●	●	●	●	●
Estonia (Tallinn)	Domestic care service (koduteenus) 24/7 general care (Õöpäevane üldhooldus)	●	●	●	●		●	●	●
Finland	Care allowance (Eläkettä saavan hoitotuki)	●	●	●	●	●		●	
	Home care services (Kotihoito and Tukipalvelut) Institutional care	●	●	●	●	●	●	●	●
France	Allocation Personnalisée d'Autonomie (APA)	●	●	●	●	● ¹			
Germany	Pflegegeld (Care Allowance) Pflegesachleistungen (Care Benefits In-Kind) Hilfe zur Pflege (Assistance for Care)	●	●	●	●	●			
Italy (South Tyrol)	Care allowance (Assegno di cura) Home care services (Assistenza domiciliare) Residential services	●	●	●	●	●			
Ireland	Home support service Nursing home support scheme	●	●	●	●	●	●	●	

Countries and subnational areas	Benefit and schemes	ADL	IADL	Cognitive state	Social & emotional needs	Health conditions	Safety & accessibility	Family/carer support	Other
Israel	Home care via the National Insurance Institute	●				●			
Japan	Long-term Insurance System (介護保険制度)	●	●	●		●			
Korea	Long-Term Care Insurance for the Elderly (노인장기요양보험제도)	●	●	●		●			
Lithuania	Social care Social assistance Institutional care Municipal support	●	●	●	●	●		●	
Luxembourg	Long-term care insurance Complément accueil gérontologique	●	●			●			
Netherlands	Social Support Act 2015 (Wmo)	●	●	●	●				
	Health Insurance Act (Zvw)					●			
	Long-Term Care Act (WLZ)	●	●	●	●	●	●	●	●
New Zealand	Home support services Residential care	●	●	●	●	●		●	●
Poland	Zasilek pielęgnacyjny Care supplement (dodatek pielęgnacyjny) ²	●	●						
	Home care services Residential care	●	●	●		●		●	
Portugal	Complement by dependency (for home care)	●							
	Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	●		●					
Slovenia	Assistance and Attendance Allowance (dodatek za pomoč in postrežbo) Home help [pomoč na domu]	●	●	●					
Spain	Ayuda al domicilio Atención Residencial Prestación económica vinculada al servicio Prestación económica de asistencia personal	●	●	● ³	●				
Sweden ⁴	Home care (hemtjänst) Insitutional care (särskilt boende)	●	●	●	●	●		●	●

Countries and subnational areas	Benefit and schemes	ADL	IADL	Cognitive state	Social & emotional needs	Health conditions	Safety & accessibility	Family/carer support	Other
United Kingdom	Attendance allowance Social care	●	●	●	●	●	●	●	
United States (California)	In-Home Supportive Services (IHSS) Medi-Cal Institutional Care	●	●	●	●	●	●	●	●

Note:

ADL: Dressing, walking across the room, getting in/out of bed, bathing/showering, eating, using the toilet.

IADL: preparing a hot meal, shopping groceries, telephone calls, taking medications, doing work around the house/garden, managing money, leaving the house independently/accessing transportation, personal laundry.

Cognitive state: Evaluates behaviour and mental condition. Social and emotional needs: Understanding of a person's need for social interaction, emotional support, and mental stimulation.

Health condition: Reviews medical history, current health issues, and medication management.

Safety and accessibility: Identifying any modifications or aids needed in the living space to ensure the person's safety and mobility.

Family or carer support: Considers the availability and involvement of external support from family or caregivers.

Other: Any additional needs not captured in the above categories.

1. In certain départements a medical certificate is requested.

2. Applicants over 75 years of age, the benefit is granted ex officio by the pension authority.

3. No scale or direct question about this, but guidance is given to the assessors to look for memory loss.

4. There is no legal framework on needs assessment or required instruments for the assessment. Some municipalities use the KATZ ADL-index or any other type of scale. Others use an administrative application form.

Source: Questionnaires, MISSOC, OECD material and web-search result see main table.

33. Assessing the comprehensiveness of the needs assessments to determine a person's eligibility for benefits and/or the generosity of public support is not straight forward. In Table 10, countries have been attributed to three levels of comprehensiveness: low, medium and high level of comprehensiveness. The following criteria were used for the categorisation:

- **Low comprehensiveness:** A country is ranked as low if its needs assessment primarily focuses on limitations with Activities of Daily Living (ADLs), such as difficulties with dressing or bathing independently. This limited focus indicates a narrower approach to assessing LTC needs.
- **Medium comprehensiveness:** A country enters this category if its assessment process considers at least six ADLs, as well as some Instrumental Activities of Daily Living (IADLs). For example, difficulties in preparing a hot meal or taking medications are some of the limitations considered. This category reflects a more inclusive approach by encompassing a broader range of functional tasks compared to the low level.
- **High comprehensiveness:** A country is ranked as very comprehensive when its needs assessment adopts a holistic approach to determine individuals' care needs. This includes conducting a broad functional assessment, including a wide range of ADLs and IADLs. Additionally, the assessment considers cognitive, behavioural, and/ or social limitations. This category refers to systems that, through their needs assessment, recognise the multi-dimensional nature of LTC needs by evaluating not only functional limitations but also cognitive, behavioural and social ones.

34. It is important to acknowledge certain limitations in the proposed approach. Firstly, the methodology employed for categorising systems may require further refinement to ensure an objective and comparable evaluation of the needs assessment scales used in the covered countries. Secondly, the scarcity of information for some countries poses challenges in accessing detailed descriptions of their respective need assessment tools, potentially impacting the accuracy of certain evaluations. For the aforementioned reasons, the results for these indicators are only indicative.

Table 10. Needs assessment comprehensiveness

Country	Comprehensiveness	Detailed needs considered
Austria	Medium	ADLs and IADLs as well as cognitive impairments
Belgium	High	all ADLs, IADLs, cognitive impairments, social life & awareness
Bulgaria	Low	6 ADLs, 2-3 IADLs
Croatia	Low	focused on impairments (ADLs only)
Czechia	Medium	6 ADLs, 7 IADLs
Cyprus	Medium	6 ADLs, 4 IADLs
Denmark	High	functional ability (Barthel index), IADLs, cognitive impairments, home conditions and possibilities of self-determination, and an indication of rehabilitative support
Estonia	High	ADLs, IADLs, psychological (mental health) and social environment
Finland	High	Assessment of the various dimensions of functional capacity, including cognitive, mental, social, and environmental factors.
France	High / medium	10 physical and mental activities, known as discriminating activities and 7 domestic and social activities, known as illustrative activities
Germany	High	Mobility, cognition and communication activities, behavioural patterns and psychological problems, self-reliance, social needs, among other
Greece	Low	No specific indicators, assessment is linked to the level of invalidity based on illnesses.
Hungary	Medium / low	ADLs, IADLs, orientation in time and space, behaviour & communication
Ireland	High	ADLs, IADLs and cognitive, includes risks within the home environment and informal care availability
Israel	Low	6 ADLs
Italy	Low	ADLs (based on KATZ scale) and cognitive disorders, employment, daily routines and social relationships
Latvia	Low	ADL, IADL, communication, behaviour, settlement of conflicts

Lithuania	Medium	Mobility, self-care, social environment, communication, nutrition, cognitive and emotional perceptions & other functions.
Luxembourg	Medium	6 ADLs, 4 IADLs
Malta	Medium	ADLs, IADLs and social state
Netherlands	High	Health problems, physical and cognitive limitations, social and home environment, quality of life and service-use & availability of informal support
Poland	Medium	All ADLs, 4 IADLs
Portugal	Low	Based on Barthel index and ADLs only
Romania	High	Functional, sensorial, and psycho-emotional indicators
Slovakia	Medium	6 ADLs, IADLs
Slovenia	High	Mobility, cognitive and communication skills, behavioural and mental health, self-care among others
Spain	High	6 ADLs, 8 IADLs & broader on decision making
Sweden	High	ICL standard: ADLs, IADLs, cognition, social relations, self-care, domestic life, communication and activities for leisure

Source: OECD social protection questionnaire and secondary national sources, MISSOC 2023.

35. As Table 10 shows, within the general categories of ADL, IADL and other needs, the specific needs considered in each assessment vary from country to country. The needs considered in the areas of ADLs and IADLs are relatively similar in most countries, while any additional needs considered differ more between countries. In Romania, for example, community home care services can support with: 1) basic activities of daily living, including ensuring personal hygiene, dressing and undressing, feeding and hydration, ensuring hygiene, transferring and mobilising, moving indoors and travelling outside and communication; 2) instrumental activities of daily living including preparing food, shopping, housekeeping and laundry, administration and management of property, medical treatments, leisure activities and socialisation. These are common ADLs and IADLs that most countries include in their needs assessments. Some countries define needs at this relatively broad level, while others go into more detail, like e.g. Cyprus, what asks older people specifically about activities such as sweeping and mopping, dusting, bathroom and toilet cleanliness, dish washing, laundry, ironing, cooking / diet, tidying, general cleaning / garbage.

36. In addition to ADLs and IADLs, Romania considers visual, hearing and speaking ability, orientation, memory, judgement, behaviour (e.g. hypoactivity) and affective disorders such as depression. Germany additionally considers temporal and spatial orientation, taking decisions on their own account, aggressive or anxious behaviour and nightly restlessness, as part of behaviour patterns and psychological problems. Belgium via its BelRAI instrument includes danger to self and to others, addictions and dependencies or other psychiatric symptoms as well as resistance to care and inappropriate behaviour of other kinds.

Tools and scales for needs assessments

37. Nationwide standardised need assessments are in place in a number of countries such as Austria, Belgium, Czechia, France, Germany, Latvia, Lithuania, Netherlands, Portugal and Spain (see Table 2). Applying uniform criteria across the country can strengthen consistency, equity, and efficiency, by reducing regional disparities in access and care. It also streamlines administration, improves data collection for policy planning, and ensures that individuals can move between municipalities without losing benefits or facing reassessment. Some countries have more than one LTC needs assessment tool, because they target different purposes or because they are designed by and for subnational areas, such as municipalities. For example, in Austria, the national cash benefit (*Pflegegeld*) is determined by care needs and eligibility through a standardised tool with seven categories, however in-kind benefits are organised differently in each region. Portugal developed one needs assessment tool named the Integrated Bio-psychosocial Assessment Instrument for in-kind LTC in public and not-for-profit providers. In parallel, for-

profit care providers can assess needs independently. The assessment tool for the cash benefits of the social sector relies on the same instrument but has additional criteria **Invalid source specified..**

38. Locally developed needs assessments are also very common in the Nordic countries: in Sweden, there is no standard needs assessment across the country – each municipality develops their own needs assessment tool. They value the greater flexibility and responsiveness, and the possibility to tailor services to the specific needs of local populations. Similarly, in Denmark, the decision on the level of support is taken on the basis of a questionnaire established by the local authorities and in Finland the municipality grants services on the basis of an assessment of individual needs such as the RAI. In April 2023, Finland introduced the universal use of RAI (Resident Assessment instrument) by interRAI as an assessment tool for in-kind home care. InterRAI is a collaborative network of researchers and practitioners in over 35 countries that developed modules for people who are medically complex and/or people with disability. With this tool, the performance of everyday activities is assessed, as well as daily decision-making and service need. If necessary, other assessment tools measuring health and functional ability are used.

How are needs assessed?

39. As the examples show, the assessment tool used to assign people to a benefit strongly varies by country: some – like Korea, Spain, Estonia (Tallinn) or Germany – use a standardised needs assessment that translates via a point system into care levels which are matched with benefit amounts. Poland also uses a point system, though the points do not translate into a care level but in a benefit amount directly. In contrary, Italy (South Tyrol) and Austria do define care levels, but instead of using a point system, they base the level on the amount of hours of care a person is assessed to require per month. The Netherlands grant benefits based on individual care profiles and Japan uses a decision tree algorithm to determine eligibility. Czechia uses the needs assessment to determine the number of LTC needs a person has (e.g. needing help with washing or dressing), granting benefits for those with at least 3 out of 10 needs.

40. The outcome of the needs assessment is matched with benefit amounts, either in monetary value or in hours of care to be provided. Some examples for the scales and their respective benefit amounts are included in Part 3 - Generosity and coverage. Those countries that assign individuals to care levels, use one of the lowest categories of care needs to define the minimum care needs a person has to have to qualify for benefits. In most countries, individuals with lower needs than those defined in the lowest category do not receive any benefits. In some countries, the lowest category does still not entitle a person to the benefit. This is for example the case in Germany, where the lowest category grants a small “relief amount” (*Entlastungsbetrag*) of EUR125 per month and only from the second category onwards the real LTC benefit is granted. Similarly in France only four out of six categories grant entitlement to benefits. France assigns people to categories by assessing their degree of loss of autonomy and individuals in the categories 5 and 6 are considered autonomous enough to live without formal support. They do, however, have options to receive support at home with certain household tasks such as cleaning, preparing meals, doing laundry or shopping if they have limited financial capacity to pay for these services themselves. A similar logic is used in countries that do not define care levels, where e.g. a number of hours or points is attributed to a certain benefit amount and a minimum that qualifies for benefits.

41. In addition to Finland, also Germany and Estonia have changed their needs assessments over the past years. Germany now uses a needs assessment which is particularly comprehensive. The regional Health Insurance Funds or private LTC insurance companies appoint independent assessors who measure abilities in six domains and for each criterion, a point value of self-reliance ranging between 0 (full self-reliance) and 3 (full dependence to assistance) is attributed. The domains have different weights, the highest weight being for the personal care domain and the lowest for the mobility domain. The total score is used to attribute a level of severity of impairment ranging from 1 - minor impairments to 5 - most severe impairments. In 2017, Germany put more weight on dementia in its assessment tool as part of the Second Care Strengthening Act (PSG II). The objective was to redefine the definition of the need for care.

Until 2017, primarily people with physical impairments were considered to be in need of care. People with dementia were certified as having limitations, but this was associated with significantly lower benefits. The new definition of the need for care offers equal access to benefits for people with cognitive impairments and those in need of physical care. Slovenia has used the German model as a starting point to develop their own needs-assessment tool since 2016 via extensive pilots.

42. Estonia implemented a reform in 2023, to ensure that the state provides additional financial resources to municipalities, in addition to their existing funding, to support the organisation of LTC. Municipalities can use the funds provided by the state to organise both nursing home services and home care services. The support will be distributed proportionally between local government entities based on the number of people aged 65 to 84 and aged at least 85. The aim of this division is to give local authorities the opportunity to organise the entire journey of social services for the LTC of the older people. For institutional care the public authorities also cover more of the cost following the reform: While a person had to cover the full cost of accommodation of the residential home, from 1 July 2023 this cost is split between the person in need of the service and the local authority (Sotsiaalministeerium, 2023^[5]).

Table 11. Levels of care need and weighting

Countries / subnational areas	Benefit and schemes	Geographical scope	Categories/ levels of care needs	Weights and points thresholds
Australia	Commonwealth Home Support Programme Home Care Package Aged care home	Nationwide	4 levels of HCP based on care needs	No weights
Austria	Care allowance (Pflegegeld) 24-hour care	Nationwide	7 categories of LTC based on care hours per month	Eligibility set a 65h of care needed/month, expected to last for at least 6 months for LTC allowance (Pflegegeld). Benefit for 24-hour care as of category 3.
Belgium (Flanders)	Care budget for people with severe care needs	Nationwide	None	at least 13 out of 30 for the BelRAI Screener and/or at least 5.5 points on the sum of AIDL and ADL modules are entitled to the care budget.
	Care budget for the elderly with care needs	Nationwide	5 categories	Score at least 7 points of the 'self-reliance scale'. Points attributed for each function are added up and the person is assigned to a category.
	Incontinence allowance	Nationwide	None	B or C on the Katz scale and score 3 or 4 in the dependency schedule for the criterion "incontinence".
	Home care organisations	Nationwide	None	-
Canada (Ontario)	Home and Community Care	Province	None	interRAI CHA for somewhat frail or dependent clients requiring assistance for independent living. interRAI CA: Clients with short-term or less complex needs.
	Long-term Care Homes	Province	None	interRAI HC is used to assess the needs of long-stay home care clients.
Czechia	Care allowance (Příspěvek na péči)	Nationwide	4 levels of dependency based on the number of basic needs unable to perform without help.	Eligibility based on the number of basic living needs that cannot be performed, at least 3 out of 10 testing basic needs.
Denmark	Home help (Hjemmehjælp)	Municipal	None	No minimum level

Countries / subnational areas	Benefit and schemes	Geographical scope	Categories/ levels of care needs	Weights and points thresholds
	Nursing Home Housing (Plejhjembsoliger)			
Estonia (Tallinn)	Domestic care service (koduteenus) 24/7 general care (Ööpäevane üldhooldus)	Municipal	3 levels based on points	Minimum 20 out of 100 points.
Finland	Care allowance (Eläköntä saavan hoitotuki)	Nationwide	3 levels based on care amount needed	Thresholds vary across municipalities
	Home care services (Kotihoito and Tukipalvelut) Institutional care	Nationwide	None	Thresholds vary across municipalities
France	Allocation Personnalisée d'Autonomie (APA)	Nationwide	6 levels but only levels 1 to 4 give entitlement to APA	10 variables with 4 degrees (spontaneously, totally, correctly, normally)
Germany	Pflegegeld (Care Allowance) Pflegesachleistungen (Care Benefits In-Kind)		5 care levels	For each criterion in the 6 areas the degree of autonomy is measured between 0 and 3. There is also different weightings are combined to form a total value. Minimum of 12.5 out of 100 points
Ireland	Home support service Nursing home support scheme	Nationwide	None	-
Israel	Home care services	Nationwide	6 care levels	At least 5.5 hours per week, up to 30 hours per week
Italy (South Tyrol)	Care allowance (Assegno di cura)	Regional	4 levels based on the amount of care hours needed per month	-
	Home care services (Assistenza domiciliare) Residential services	Regional	None	-
Japan	Long-term Insurance System (介護保険制度)	Nationwide	7 possible categories: two support levels and care levels graded from 1-5. The first 2 levels are considered assistance level and qualify for preventive services, the other 5 are care levels	There is no specific scoring system. The level is computed based on the decision tree algorithm.
Korea	Long-Term Care Insurance for the Elderly (노인장기요양보험제도)	Nationwide	6 level of care needs based on survey.	52 items out of 90 item survey used to calculate care level. Minimum 45 out of 100 points, unless applicant displays a cognitive condition.
Lithuania	Social care Social assistance	Nationwide	2 levels based on the amount of care hours needed per day	-
	Institutional care Municipal support	Nationwide	None	-
Luxembourg	Long-term care insurance Complément accueil g�rontologique	Nationwide	15 levels based on minutes of care per week	at least 3.5 hours per week and if the user's dependency condition is likely to last longer than 6 months or to be irreversible.
Netherlands	Social Support Act 2015 (Wet Maatschappelijke Ondersteuning 2015, Wmo)	Municipal	None	-
	Health Insurance Act (Zorgverzekeringswet, Zvw)	Provider	None	-
	Long-Term Care Act (Wet Langdurige Zorg, WLZ)	Nationwide	No specific care levels but based on the assessment, creating of 'care profiles'	-

Countries / subnational areas	Benefit and schemes	Geographical scope	Categories/ levels of care needs	Weights and points thresholds
New Zealand	Home support services Residential care	Nationwide	None	-
Poland	Zasiłek pielęgnacyjny Care supplement (dodatek pielęgnacyjny) Home care services Residential care	Nationwide	None	people with 0 to 40 points on the Barthel scale receive LTC
Portugal	Complement by dependency (for home care)	Nationwide	2 degrees of dependency	-
	Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	Nationwide	None	-
Slovenia	Assistance and Attendance Allowance (dodatek za pomoč in postrežbo)	Nationwide	3 levels based on care needs	-
	Home help (pomoč na domu)		None	-
	Nursing home care		4 levels	-
Spain	Ayuda al domicilio Antención Residencial Prestación económica vinculada al servicio Prestación económica de asistencia personal	Nationwide	3 levels	score is the sum of the weights of the tasks for which the individual has difficulty, multiplied by the degree of supervision required and the weight assigned to that activity. Minimum 25 points (highest is 100).
Sweden ²	Home care (hemtjänst) Institutional care (särskilt boende)	Regional	None	Thresholds vary across regions
United Kingdom (England)	Attendance allowance	Municipal	2 levels level based on care needs	-
	Social care	Municipal	None	Thresholds vary across municipalities
United States (California)	In-Home Supportive Services (IHSS)	County	6 ranks to determine how many hours per activity are required	Functional Index Ranking, a point-based system, indicates the level of assistance
	Medi-Cal Institutional Care	County	None	-

Note: Standardisation level: If the assessment procedures and the criteria applied are the same nationwide, or just at regional or local level.

Weight/Points threshold: how the eligibility for benefits is determined or measured.

Categories or Levels: the number of levels or categories that are used to classify beneficiaries, such as levels of care or dependency based on specific criteria.

Source: OECD analyses based on Long-Term Care Social Protection questionnaire.

43. An effective needs assessment can be created by relying on a case-mix classification based on various evidence-based scales. The outcomes of these scales measure different aspects of LTC and weighing them can help create groups of statistically related patients. An alternative is creating a case mix classification with a new scale which allows designing a tool based on country-specific characteristics. Various questions are typically grouped together to create the scales. The specific aspects of LTC usually covered are:

- ADL
- IADL
- Cognitive performance scale
- Communicative Scale
- Pain scale
- Aggressive behaviours scale

- Pressure ulcer risk scale
- CHES- Changes in Health, End-stage disease and Signs and Symptoms
- MAPLe- Method for Assigning Priority Levels
- Deaf/blind severity index

44. Defining the relative weights of the scales in the case mix classification requires knowledge of the drivers of home care utilisation (e.g. most frequent needs or the services mostly used). This enables predicting utilisation and cost accurately, which is paramount for the related payment system. Japan conducted the analysis of the drivers of care utilisation to determine the most appropriate case-mix classification and develop their scale. Such analysis allowed Japan to tailor the tool based on the specificities of the Japanese population and system and prevented the mere replication of the American case mix classification, which would have focussed too much on physical therapy. Researchers in the Netherlands have also conducted a similar analysis to serve as input for the development of a new payment system for home care (OECD, 2022^[6]).

45. A similar emphasis on tailoring tools to specific needs is seen in Ontario, Canada, where the interRAI system provides a diverse range of assessment tools designed to meet different care settings and patient profiles. InterRAI is a collaborative network of researchers and practitioners in over 35 countries that developed modules for people who are medically complex and/or people with disability. For example, the interRAI CHA is used for community-based care, the interRAI CA for short-term or less complex cases, and the interRAI HC for long-stay home care clients. These tools work in unison to form a comprehensive system that addresses the full continuum of LTC needs. Ontario's upgrade from RAI-Home Care (RAI-HC) to the more advanced interRAI-Home Care (interRAI-HC) reflects a move towards greater precision and comprehensiveness in assessments across both home and institutional care. Additionally, the interRAI-HC includes specialised features such as the CaRE scale, which helps identify caregivers at high risk of caregiver burden, and the CRISIS algorithm, which assesses an individual's likelihood of requiring LTC placement within 90 days. These innovations allow for more targeted interventions and resource allocation, further enhancing the effectiveness of Ontario's home care system.

46. Another consideration countries have to make is whether to include digital tools as part of the needs assessment. Such a tool gives access to a wealth of information relevant to key questions facing providers and decision makers across the health and social services. If digital assessments are regularly carried out, care providers could more easily evaluate the effectiveness of care plans and public agencies could evaluate the quality of care. The purposes of a digital needs assessment include:

- Eligibility for public support
- Care planning for providers (provided that the assessment takes place on a regular basis)
- Evaluation of care effectiveness (thanks to longitudinal microdata)
- Monitoring of providers' care quality indicators
- Monitoring of care quality at the municipal and national levels

47. While many digital assessment instruments exist, InterRAI seems to be one well-considered (RAI stands for Resident Assessment Instrument). However, there are two fundamental pre-requisites to have a digital needs assessment: an excellent IT system and trained staff.

Assessors of long-term care needs

48. In Austria, Croatia, Czechia, France, Germany, Greece, Ireland, Latvia, Poland, Portugal, Romania, the Slovak Republic and Spain, the assessment is performed by a health professional or a multidisciplinary team composed of at least one health professional (see Table 3). In Luxembourg, a social worker or a health professional can perform the assessment. In other countries like Denmark, England, Estonia, Netherlands and Sweden, social workers perform the assessment. In Japan, due to a lack of

nurses, the needs assessment tool and the training were designed to ensure that the assessors would not require medical nor social service expertise. However, the mayor of municipalities appoint doctors, nurses and social workers to form a Nursing Care Needs Certification Board, to review and validate the initial assessment, considering the applicant's primary care physician's statement and the notes written by the assessor during the home visit **Invalid source specified**. Slovenia plans that the Health Insurance Institute will train a broad range of future assessors and will supervise them. In Croatia, since 2017, a panel of experts determines a person's type and degree of disability, which is chaired by a physician and is composed of at least one physician and one expert on disabilities who have a minimum of two years' experience each.

49. The shortage of healthcare professionals is a critical issue to consider. For example, in Australia, non-clinical personnel is used to assess eligibility for the Commonwealth Home Support Programme, which is designed to assist individuals who need a lower level of support to maintain independent living. This allows for clinical personnel to focus on more complex cases, where their expertise is needed most. In Denmark, municipalities address this issue by hiring evaluators from non-clinical backgrounds but with practical experience in the care sector. Similarly, Sweden employs trained social workers to conduct assessments, ensuring that assessments are handled efficiently without placing additional strain on the healthcare workforce.

50. Other countries have developed alternative responses to this shortage, relying on a general practitioner's issuing a medical certificate, relying heavily on an individual's existing medical records. Authorities then base their assessment of care needs on this documentation. If the provided evidence is insufficient to determine the level of need, a formal needs assessment may be conducted. This approach is used for certain benefits in countries such as the England, UK (Attendance Allowance), Finland (Care Allowance), and Portugal (Complement by Dependency).

51. Independent bodies assessing the LTC needs for all or some of the services exist for example in Germany, contracted by the LTC insurance; in Australia for residential aged care funding; and in the Netherlands by the Care Needs Assessment Centre (*Centrum Indicatiestelling Zorg*), an independent government body. Independent assessors can provide more objective, consistent, and unbiased evaluations, ensuring fair and equitable access to care based solely on an individual's needs.

52. Some countries build into the system the obligation to reassess a person's needs on a regular basis. The LTC needs of older people is reassessed every two years or after a marked deterioration in Japan (Japan Health Policy NOW, n.d.^[7]). In Ontario, the assessment is carried out every six months for home care. A study found that 80% of home care clients have significant clinical changes in health status within 6 months and that the cost of assessment represents 1.55% of the home care cost (CAN 23.6 million vs. CAN 1526.5 million) (Kinsell et al., 2020^[8]). However, as shown in Table 12, the frequency of reassessments varies widely across countries, ranging from several months to as long as five years. Despite this diversity, the majority of countries mandate reassessments at least once a year.

The location where the needs assessment takes place

53. Some countries provide a range of options where the needs assessment should take place, others give all the flexibility to those who conduct it (see Table 12). Independent of who decides on the location of the assessment, ensuring flexibility to cater to less mobile older people is essential to ensure equal access to the public benefits. In Australia, for example, face-to-face is the preferred approach and must take place where possible. Exceptions can be made e.g. in remote areas, where it can be conducted over the phone or via telehealth. The face-to-face assessments can take place in the older person's home, the carer's home, a community setting, at the residential aged care service, in a hospital, or other health settings.

54. Home-based assessments are predominant across many countries, with the option to conduct assessments in nursing homes, hospitals, or rehabilitation centres when necessary. While a few countries require individuals to visit a specialised governmental centre for the assessment, these countries often provide home visits for applicants who are unable to travel like it is the case in Italy, Portugal and Slovenia.

55. Online needs assessment were widely used during the COVID19 pandemic. Four years later, however, almost all countries have reverted back to the previous arrangements to ensure that an in-person assessment can take place. Only two countries still allow for virtual needs assessment under very specific circumstances. The first one is Australia, where the option to conduct a virtual needs assessment was maintained exclusively for individuals who live in very remote areas. Given Australia's geographical conditions, this is expected to improve access to benefits and reduce waiting times. The second country is Italy, where the virtual assessment is kept as a very last resort when all other options of in-person meetings could not be arranged.

Table 12. Overview of Needs Assessment Process for Long-Term Care Benefits

Countries and subnational areas	Benefit and schemes	Assessor	Setting of the assessment	Approving governmental body (level)	Min reassessment frequency
Australia	Commonwealth Home Support Programme	RAS team ¹ : Non-clinical	Home and institutional	ACAT delegate (State)	12 months
	Home Care Package Aged care home	ACAT assessors ¹ : Healthcare professional (medical, nursing, psychologist, occupational therapist, physiotherapist) or social worker.	Home and institutional	ACAT delegate (State)	12 months
Austria (Viena)	Care allowance (Pflegegeld) 24-hour care Fonds Soziales Wien	Medical expert	Home and institutional	Regional social insurance institutions (Regional)	Not specified
Belgium (Flanders)	Care budget for the elderly with care needs	Doctor or evaluator from the Directorate-General for Persons with Disabilities.	Govt. centre	Health insurance fund (Regional)	Not specified
	Care budget for people with severe care needs	Trained individuals on the use of BelRAI Screener, which may be from the family care services, the Public Centre for Social Welfare, and the Social Work Services that are affiliated with the health insurance funds.	Govt. centre	Health insurance fund (Regional)	Not specified
	Home care organisations	A trained individual from the family care services	Home	Service provider (Local)	12 months
Canada (Ontario)	Home and Community Care Long-term Care Homes	Care coordinator: Social worker or healthcare professional (medical, nursing, occupational therapy, physiotherapy, speech therapy)	Home and institutional	Ontario Health atHome (Province)	Not specified
Czechia	Care allowance (Příspěvek na péči)	Social worker and medical examiner	Home and institutional	Regional branch of Labour Office	Not specified
Denmark	Home help (Hjemmehjælp) Nursing Home Housing (Plejehjemsboliger)	Evaluators are not formally trained, but have experience in the care sector	Home	Municipal council (Local)	12 months
Estonia (Tallinn)	Domestic care service (koduteenus)	Social worker	Home	Social Welfare and Health Care Department (Local)	12 months
	24/7 general care (Ööpäevane üldhooldus)	Social worker, medical doctors and nurses. For more complex cases, a rehabilitation team (social worker, psychologist, occupational therapist, psychiatrist and psychiatric nurse, patient experience advisor).			
Finland	Care allowance (Eläkettä saavan hoitotuki)	Doctor (issue a medical certificate)	Institutional	Kela ((National)	Not specified
	Home care services (Kotihoito and	Social worker or a healthcare professional	Home	Wellbeing Services County	Not specified

Countries and subnational areas	Benefit and schemes	Assessor	Setting of the assessment	Approving governmental body (level)	Min reassessment frequency
	Tukipalvelut) Institutional care			(Regional)	
France	Allocation Personnalisée d'Autonomie (APA)	Medical-social team (l'équipe medico-sociale, EMS): doctors, social workers, nurses, occupational therapists, and administrative staff. Coordinating physician at the nursing home.	Home	Départements (Local)	Not specified
Germany	Pflegegeld (Care Allowance) Pflegesachleistungen (Care Benefits In-Kind) Hilfe zur Pflege (Assistance for Care)	Medical service or independent assessors: Doctors and specialist nurses.	Home	Long-term care insurance fund (National)	Not specified
Ireland	Home support service	A healthcare professional: Public health nurse, physiotherapist or occupational therapist.	Not specified	Home Support Manager	Not specified
	Nursing home support scheme	A healthcare professional: geriatrician, public health nurse, or occupational therapist.	Not specified	Nursing Homes Support Offices	Not specified
Israel	Home care via the National Insurance Institute	Assessor from the National Insurance Institute (geriatrician for people above 90 years old)	Home	National Insurance Institute	Not specified
Italy (South Tyrol)	Care allowance (Assegno di cura)	Evaluation team: a nurse and a social worker.	Govt. centre or home	Provincial service for the evaluation of non-self-sufficiency	Depending on the assessment outcome
	Home care services (Assistenza domiciliare) Residential services	The director/manager of the care provider or facility with the biding opinion from a designated committee that may include technical care manger, a doctor, direct care staff. A nurse and a social worker must always be present in the committee.	Not specified	Local health authority	
Japan	Long-term Insurance System (介護保険制度)	Municipal officer: certified health/social care worker with special training from the municipal office.	Home	Municipal governments (Local)	Not specified, but up to 48 months ²
Korea	Long-Term Care Insurance for the Elderly (노인장기요양보험제도)	National Health Insurance officer	Home	Needs Assessment Committee (Local)	2-4 years, depending on need levels
Lithuania	Social care Social assistance	Medical professionals for nursing and medical care, and social workers for social services.	Not specified	Municipality (Local)	Not specified

Countries and subnational areas	Benefit and schemes	Assessor	Setting of the assessment	Approving governmental body (level)	Min reassessment frequency
	Institutional care Municipal support				
Luxembourg	Long-term care insurance Complément accueil gérontologique	Applicant's doctor issues a medical report, and a review by a doctor or a health professional (nurse, psychiatric nurse, occupational therapist, psychologist or social worker) from the State Office for Assessment and Monitoring of the long-term care insurance (AEC) is conducted.	Govt. centre, home or institutional	National Health Fund (National)	Varies depending on circumstances
Netherlands	Social Support Act 2015 (Wet Maatschappelijke Ondersteuning 2015, Wmo)	Wmo consultants or social neighbourhood teams (sociale wijkteams, SWTs)	Home	Municipality (Local)	Not specified
	Health Insurance Act (Zorgverzekeringswet, Zvw)	District nurses	Not specified	District nurse and health insurance	Not specified
	Long-Term Care Act (Wet Langdurige Zorg, WLZ)	Applicant's doctor and if required the Care Needs Assessment Centre (Centrum Indicatiestelling Zorg, CIZ)	Institutional or home	CIZ (National)	Not specified, but up to 5 years
New Zealand	Home support. services Residential care	Trained healthcare professionals (doctors, nurses, physiotherapists, etc) from the New Zealand Assessment Service Co-ordination association.	Home or Govt. centre	New Zealand Assessment Service Co-ordination association (NASC) (National)	Not specified, but no longer than 3-year interval
Poland	Care supplement (dodatek pielęgnacyjny)	User's doctor issues a medical certificate and authorised doctors of the Social Insurance Institute reviews application	N/A	Social Insurance (National)	Not specified
	Zasilek pielęgnacyjny	Certified physician working with the District Disability Assessment Team	Govt. centre or institutional	District Disability Assessment Team (Local)	Not specified
	Home care services (usługi opiekuńcze w miejscu zamieszkania) Residential care (Dom pomocy społecznej)	User's doctor issues a medical certificate and a social worker conducts a needs interview	Home	Local government's social welfare centre (Local)	Not specified
	Care supplement (dodatek pielęgnacyjny)	User's doctor issue medical certificate and ZUS medical board reviews	Institutional	Pension authority (National)	
Portugal	Complement by dependency (for home care) Estrutura residencial para pessoas idosas (ERPI) (for institutional care)	User's doctor issues medical certificate and a doctor from the Social Security Institute (medical board) reviews	N/A	Social Security Institute (National)	Not specified
Slovenia	Assistance and Attendance Allowance (dodatek za pomoč in potrežbo)	Medical experts of the disability commission located at the Institute for Pension and Disability Insurance	Govt. centre or home/institutional	ZPIZ (National)	Not specified

Countries and subnational areas	Benefit and schemes	Assessor	Setting of the assessment	Approving governmental body (level)	Min reassessment frequency
		(zavod za pokojninsko in invalidsko zavarovanje, ZPIZ).			
	Home help [pomoč na domu] Nursing home care	Service providers	Home	Service provider (Local)	Not specified
Spain	Ayuda al domicilio Atención Residencial Prestación económica vinculada al servicio Prestación económica de asistencia personal	Assessors with a social-health profile with specific training in the application assessment tool	Home or institutional	Regional Social Services (Regional)	Not specified
Sweden ²	Home care (hemtjänst) Institutional care (särskilt boende)	Welfare officer: trained social worker.	Home	Municipality (Local)	12 months
United Kingdom (England)	Attendance allowance	User's doctor provides medical information about the user	Institutional	Department for Work and Pensions (National)	Not specified
	Social care	Social worker or occupational therapist	Home	Municipal council (Local)	Not specified
United States (California)	In-Home Supportive Services (IHSS) Medi-Cal Institutional Care	Social worker	Home	County social services departments (Regional)	12 months

Note: 1. As part of the Single Assessment System, these teams/assessors will transition into the Single Assessment Workforce from late 2024. 2. The validity lasts 6 months for the first time, 12 months after renewal, then up to 48 months according to municipal rules. Assessor: The types of professionals or teams responsible for assessing an individual's need for care services. Setting: Home (applicant's home, carer's home), Institutional (hospitals, clinics, nursing home), or Govt. Centre (official government-operated centres specifically designed to conduct needs assessments).

Approving governmental body (level): The governmental entity responsible for approving the benefit.

Source: OECD analyses based on Long-Term Care Social Protection questionnaire.

3. Generosity and coverage

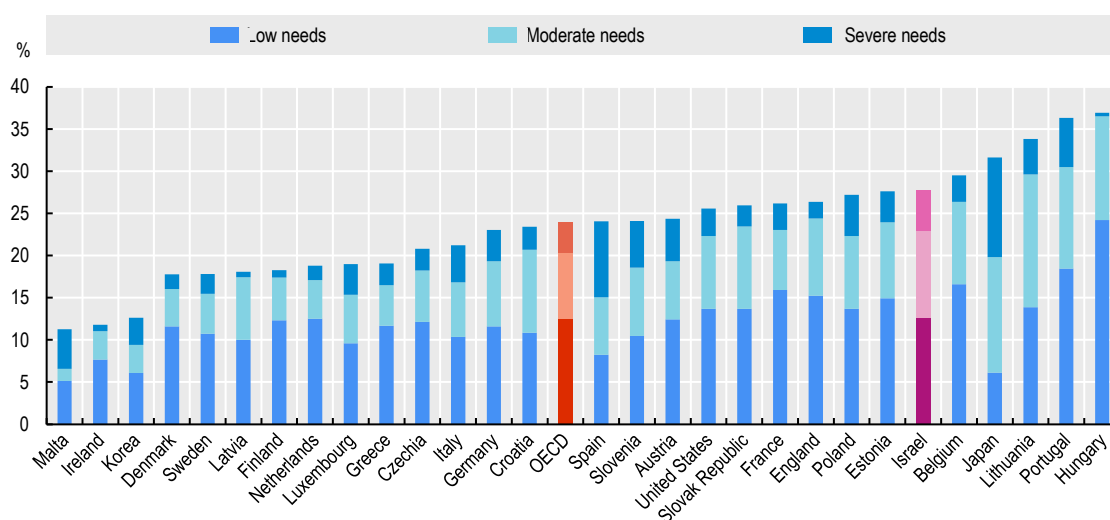
LTC needs are prevalent with nearly one in four older people displaying needs

56. The OECD has developed a series of typical cases to compare the prevalence of LTC needs, as there is no generally accepted or internationally standardised definition of LTC needs available to date – see Box 1 and OECD (2024^[9]) to consult the methodology. Based on this cross-country comparable measurement of LTC needs, it is estimated that 23.9% of older people have low (12.5%), moderate (7.8%) or severe (3.6%) LTC needs on average across OECD countries (Figure 1).

57. Age structures and life expectancy certainly have an impact on the prevalence of LTC needs. For instance, Japan has the largest share of people aged 80 years and over as well as the longest life expectancy (OECD, 2023^[10]), which may explain the highest share of older people with severe needs (11.8%). However, ageing and life expectancy are not the sole determinants: e.g., France and Greece also have higher shares of those aged 80 years and over, but their shares of those with severe needs are below the OECD average, standing at 3.2% and 2.6%, respectively.

58. Israel has a high prevalence of LTC needs despite having a younger population structure. Israel's prevalence of needs is higher than the OECD averages at every level of care needs, with the estimated shares of low needs (12.7%), moderate needs (10.3%) and severe needs (4.8%) summing up to a total of 27.8% (Figure 1). This is somewhat incongruous with the fact that Israel is a relatively younger country among the 38 OECD Member States: its population shares of 65 years and over and of 80 years and over rank 34th and 33th, respectively, whereas its life expectancy is the 11th highest. It suggests that older individuals in Israel are more likely to be dependent as they grow old.

Figure 1. Share of older people with low, moderate, and severe LTC needs



Note: The figure includes an unweighted average across 27 OECD countries. Those with low, moderate and severe needs require 6.5, 22.5 and 41.25 hours of professional long-term care, respectively.
Source: (OECD, 2024^[9]).

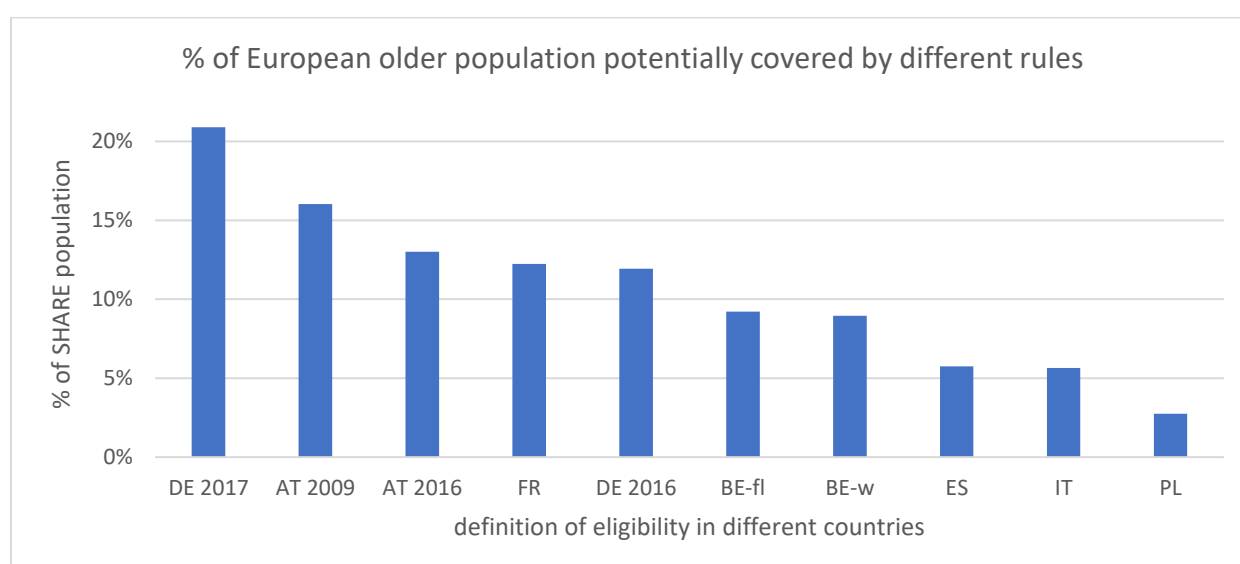
The coverage of people potentially supported by public long-term care systems depends greatly on eligibility rules

59. Even though close to one in four people aged 65 and over are potentially in need of formal or informal care on average across countries, the extent to which such individuals are (or should be) potentially eligible to receive public benefits and services is chiefly reliant on how eligibility criteria and needs assessments thereof are devised and implemented in each country. Those displaying needs (i.e. needs prevalence), those displaying the needs that are supposed to be identified and measured by public LTC systems through needs assessment, and those who eventually manage to access to public LTC benefits and services (to be discussed in the next section) are not mutually exclusive but distinct from one another in the first place. Policymakers design eligibility criteria to capture people in need of care, through a balancing act with measurement and budgetary constraints, ultimately deciding the inclusiveness of these public systems.

60. Carrino *et al.* (forthcoming^[11]) compute potential coverage – the share of those who should be granted needs according to eligibility criteria within the population – to measure the inclusiveness of public LTC systems in 8 European countries and subnational regions. The overall results indicate that potential coverage (or inclusiveness) varies greatly across countries (see Figure 2). German rules (as recently reformed since 2017) cover around 20% of the sample and are more inclusive than rules in any other country, as well as the older German rules. The 2017 German reform lowered the minimum level of vulnerability that would give access to benefits, which explains why the potential coverage rate increased after 2017.

61. In contrast, the Austrian government has also reformed their eligibility rules over recent years. Unlike Germany, Austrian reforms aimed at tightening access to LTC benefits, by increasing the minimum number of hours of care required to become eligible. The more recent definition of LTC needs in Austria would cover around 12% of older people, compared to 16% under the old rules. Moreover, compared to the legislations, the LTC coverage rates in the European population would be significantly lower under the LTC rules in Belgium (Wallonia and Flanders), Spain, Italy or Poland.

Figure 2. Potential coverage rate for an average of the European population aged 65+



Note: The potential coverage embedded in different long-term care rules, defined as the percentage of older Europeans who would be eligible to long-term care benefits according to the definitions of need of various European countries. Each bar therefore does not represent the actual use of care in each specific country, but rather the share of older Europeans who would be eligible to long-term care according to the rule of each specific country. The European population is made of a sample of 19880 respondents aged 65+, interviewed in the 6th wave of SHARE (2016) in Austria (AT), Belgium-Flanders (BE-fl), Belgium-Wallonia (BE-wa), Czech Republic (CZ), France (FR), German (DE), Italy (IT), Poland (PL) and Spain (ES). The graph reports multiple time points for the eligibility rules in Austria and Germany, to capture recent legislation changes. Source: (Carrino et al., forthcoming^[11])

The stringency of eligibility rules reflects trade-off between inclusivity and generosity

62. The inclusiveness of public systems depends not only on the size of eligible population (i.e. potential coverage) but also on the actual number of people whose needs are acknowledged through needs assessment (i.e. effective coverage). As budgetary pressure intensified in the wake of the Global Financial Crisis 2007-08, a number of OECD countries were implicitly or explicitly pushed to reinforce fiscal sustainability by targeting limited public resources at people with severe needs and people with low income, which created tension between the inclusiveness of eligibility rules and the generosity of public benefits and services (Ranci et al., 2019^[12]). A prime example of balancing inclusiveness and generosity is means-testing, as it aims to achieve a more sustainable cost-sharing mechanism by adjusting benefits depending on the applicant's income and/or wealth but to a lesser degree coverage – see discussions in Section 1.

63. To give an overview of effective coverage, Table 13 presents acceptance and rejection rates of applications for LTC needs assessment in 5 OECD countries with well-established long-term care systems: Japan, Korea, the Netherlands, Spain and the United Kingdom (England). Rejection rates vary greatly by country, ranging from a low of 2.5% in Japan to a high of 30% in England.

64. In Japan, a small number of rejection cases most likely reflects the ease of “rationing” public benefits and services. Although the final rejection rate may be 2.5%, the initial computer-based assessment indicates a rejection rate of 6.2%. However, more than half of these initially rejected cases were subsequently revised and assigned to the two bottom need grades (Support Levels 1 and 2). For these grades, local authorities can easily ration public services because: 1) public services for these grades have been arranged under local authorities' budgets (not with the LTC insurance funds) since April 2017; and 2) they can assign care managers who can design care plans that do not request a significant amount of public services. In fact, 40% of people assigned at these two lowest grades did not receive any public benefits or services, whereas the overall non-usage was 31%. From a perspective of rationing, it is understandable that Korea's rejection rate is slightly higher than the Japanese one. In the Korean LTC Insurance for the Elderly, people who are granted needs are free to choose private care providers and public benefits will be almost automatically provided, and therefore it is more challenging to ration benefits.

65. In England, on the other hand, local councils are under severe budgetary pressure and thus both need test and means test serve as a regulating valve for the already suppressed supply of public care services (Bancalari and Zaranko, 2024^[13]).

Table 13. LTC needs assessments for public benefits and services can be exclusive

Number of applications, acceptances and rejections

Country	LTC system and reference period	Applications	Acceptances (acceptance rate)	Rejections (rejection rate)
Japan	LTC Insurance System, April 2021 to March 2022	1,450,950	1,415,260 (97.5%)	35,690 (2.5%)
Korea	LTC Insurance for the Elderly, 2023	1,238,495	1,097,913 (88.6%)	140,582 (11.4%)
Netherlands	Wet langdurige zorg, (WLZ), 2015-17	163,551	Approx.. 136,300 (83.4%)	Approx.. 27,200 (16.6%)

Spain	System for Autonomy and Dependence Care, 2024	2,130,206	1,606,457 (75.4%)	523,749 (24.6%)
United Kingdom (England)	Adult Social Care, April 2022 to March 2023	1,969,810	1,379,675 (70.0%)	590,135 (30.0%)

Note: For Japan, only new applicants are included. For the United Kingdom, applications which had been submitted by those who were subsequently deceased before decision are removed from the total number of applications.

Source: Long-Term Care DB Open Data (Ministry of Health, Labour and Welfare, Japan), Long-Term Care Insurance Statistics (National Health Insurance Service, Korea), Focus on Access to the Long-term Care Act (De Algemene Rekenkamer, Netherlands), Statistical Information on the System for Autonomy and Dependence Care (Ministry of Social Rights, Consumer Affairs and 2030 Agenda, Spain), Adult Social Care Activity and Finance Report, England 2022-23 (NHS England, United Kingdom).

Public support varies greatly across the OECD, but tends to be greater for older people with more severe needs

66. In the absence of a universally accepted international measure of LTC needs, it is very difficult to compare eligibility thresholds from needs assessments and generally the comprehensiveness of LTC needs covered by social protection across Member States. As LTC is an evolving policy field, the comprehensiveness of social protection coverage has generally been extended over the last decades. Germany, for example, introduced a new definition of long-term-care needs in 2017 which allows to appropriately take into account the need of people with dementia and hence improved access to benefits for this group of beneficiaries.

67. To make meaningful international comparisons of the level of comprehensive coverage from such social schemes in different countries, a set of “typical cases” of LTC needs were defined that measures the effectiveness of social protection for LTC in old age. LTC needs include help with activities such as washing and getting dressed – grouped under what is referred to as personal care, or Activities of Daily Living (ADLs) – as well as housekeeping tasks, like cleaning and shopping – grouped under what are known as Instrumental Activities of Daily Living (IADLs). As people become more dependent, they may also find it difficult to maintain social relationships and participate in their community. They may need help with social activities, for example attending a community club or going out for a walk. The typical cases are based on number of hours of need for help with ADLs, IADLs, and social activities (see Box 1).

Box 1. Typical cases of LTC needs

As there is no single internationally accepted and standardised definition of what constitutes LTC needs, it is not possible to make meaningful comparisons across countries and subnational areas using administrative data on LTC recipients and out-of-pocket spending, as differences in eligibility, scope and depth will all be confounded. As such, a set of eight typical cases of LTC needs were developed to describe an older person in terms of the types and severity of LTC needs, and the professional services they would require. These typical cases are based on number of hours of need for help with ADLs, IADLs, and social activities, and span different levels of care severity (low, moderate, and severe) and different ways in which these needs can be met (professional home care, informal care, and institutional care). Low, moderate, and severe needs correspond to 6.5, 22.5 and 41.25 hours of care per week, respectively. Information was collected from countries on what would be the total costs of meeting the needs described in the typical cases, available LTC benefits and schemes, and rules that determine the level of support depending on the older person's income and wealth.

Source: (Oliveira Hashiguchi and Llana-Nozal, 2020^[14]), (OECD, 2024^[9])

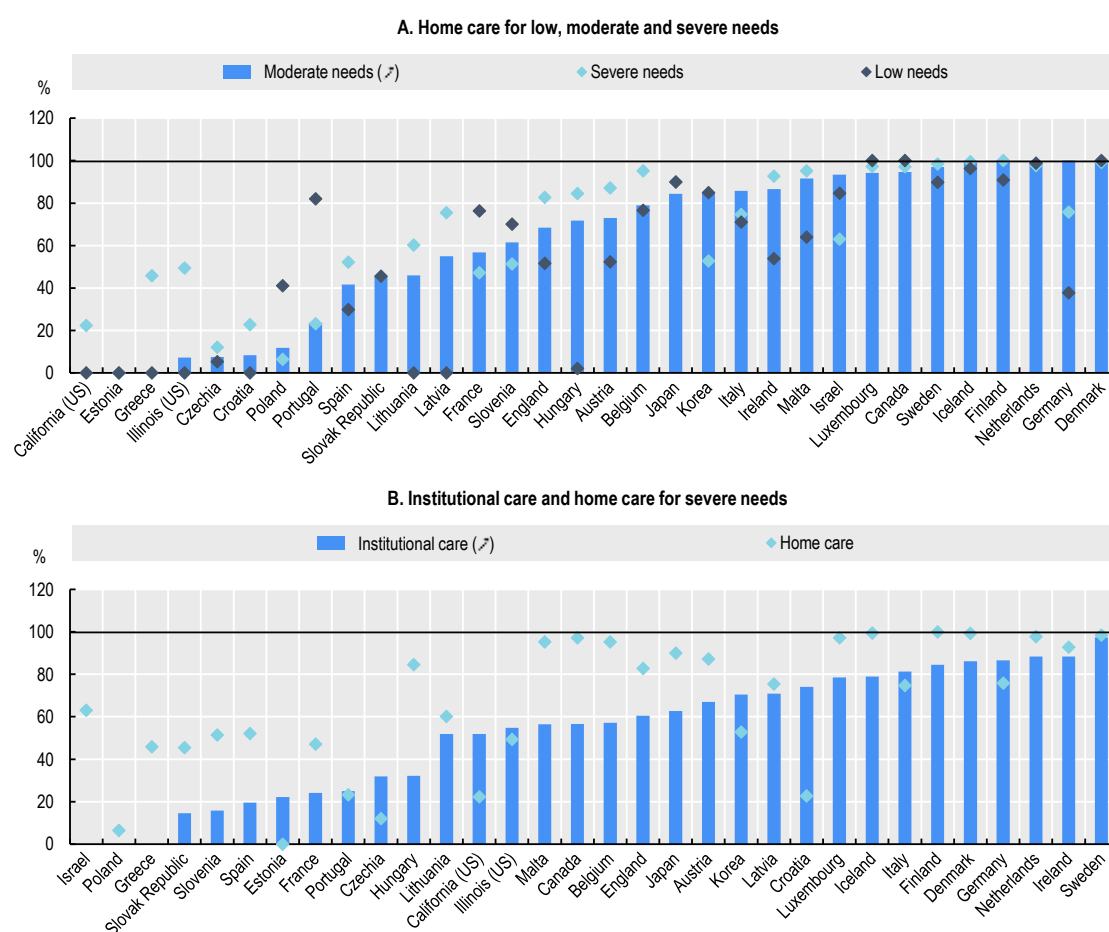
68. Public support varies greatly across the OECD. All OECD countries covered in this report provide some public support for individuals with LTC needs. For a person with severe needs, public social protection systems cover more than 50% of the costs in two thirds of countries and over 90% in one third of them and one-third cover less than 50% of the costs. Similar proportions hold for those with moderate needs. The highest generosity for all levels of care need is granted by systems in the Nordic countries (Denmark, Finland, Iceland, Sweden), as well as Luxembourg, Canada (Ontario) and the Netherlands. Generosity is more limited for a person with any level of needs and median income in Estonia, Czechia and Croatia.

69. Most support is granted to individuals with more severe needs, for whom care is usually more expensive and more likely to be unaffordable (see Figure 3). In the US (California and Illinois), Croatia and Greece, public support is limited for low and moderate needs, but more generous for people with severe needs. Exceptions include six countries where the relative generosity is greater for individuals with low needs than for those with more severe needs. Limits to the number of hours of care that can be covered through public LTC benefits can contribute to this pattern, especially in France and Slovenia. In Latvia and Lithuania, social protection is more generous with individuals with median income but only when they have at least moderate needs.

70. On average, the generosity of public benefits is greater for care provided at home, compared with institutional care (Panel B). Around 65% of home care costs are covered in the OECD, while the average covered cost of institutional care is ten percentage points lower. By contrast, the share of costs covered by public LTC systems is higher for institutional care in Estonia, Portugal, Czechia, the United States (California and Illinois), Korea, Croatia, Italy, Germany and France.

Figure 3. Share of total LTC costs that would be covered by public social protection

For individuals with median income and zero wealth



Note: Low, moderate, and severe needs correspond to 6.5, 22.5 and 41.25 hours of care per week, respectively. An older person with severe needs receiving LTC at home is assumed to live with a spouse who can provide 24-hour supervision, help with taking medicines, and manage the finances, but cannot provide any other ADL/IADL care. Data on the cost of institutional care for severe needs in Israel is missing.

Source: OECD analyses based on the Long-Term Care Social Protection questionnaire, the *OECD Income Distribution Database*, and the *OECD Wealth Distribution Database*.

4. Case studies: reforms in long-term care systems

The Netherlands

A major reform in 2015 transferred responsibilities of LTC provision to local actors. Since the late 1960s, the Dutch LTC services have been funded and administered under the *Exceptional Medical Expenses Act (Algemene wet bijzondere ziektekosten: AWBZ)*, one comprehensive LTC insurance scheme mainly funded by income-related contributions. The scheme was designed to provide care to the older people, to those with mental and physical disabilities, and to chronic psychiatric patients at home or in institutions. The implementation was ensured by Regional Care Offices, run by a single health insurer. This covered 95% of public spending, while the Social Support Act 2015 (*Wet Maatschappelijke Ondersteuning: WMO*) covered the remaining 5% and was managed by municipalities. The Dutch LTC

system comprises since 2015 three sources of funding under three different legislations, with varying sets of rules.

Since then, for almost two decades, political discussions have focussed on the unsustainability of a system increasingly viewed as supply-driven and putting clients in a mainly dependent role (Marse and Jeurissen, 2016^[15]). As AWBZ became too large, a reform process started with four key goals: (i) rein in spending growth and safeguard fiscal sustainability, (ii) improve quality of services, (iii) shift focus from residential to non-residential care and (iv) make individuals and their communities more responsible for care provision:

1. Care in nursing homes and intensive home care for people with high needs is regulated by the LTC Act (*Wet langdurige zorg: WIZ*). To access these services, individuals undergo an assessment by the Care Needs Assessment Centre (*Centrum Indicatiestelling Zorg: CIZ*), an independent agency. CIZ evaluates the person's needs and sends an assessment and entitlement decision to both the individual and the regional Care Office (*Zorgkantoor*). Once notified, the Zorgkantoor contacts the individual to arrange appropriate care services. It oversees the quality of nursing homes and home care providers based on standards set by the Healthcare Inspectorate. Care is contracted annually under a set of tariffs negotiated between unions and providers on a yearly basis, within limits set by the Dutch Healthcare Authority (*Nederlandse Zorgautoriteit: NZA*). This system is publicly funded and the government can increase funding if necessary. Eight main health insurance companies administer the system, each responsible for managing funds for care in their respective regions, regardless of the client's specific health insurance provider. Individuals may be required to make income-based co-payments for their care.
2. General home care services are regulated under the Health Care Act of 2006 (*Zorgverzekeringswet: ZVW*). Home care services have distinct rules and tariff structures for nurses and other healthcare professionals, such as general practitioners and physiotherapists. Ten main health insurance companies operate in the market, funded by compulsory premiums paid by the entire population. In addition, employers contribute a premium, collected by the Dutch National Bank and distributed to the health insurance companies based on the distribution of insured people according to socio-economic indicators such as age.
3. Domestic help for home care, home adaptation, transportation and help for informal carers is regulated by the Social Care Act 2007 (*Wet maatschappelijke ondersteuning: WMO*). The Act was reformed in 2015 to transfer more responsibilities to the municipal governments, that carry the responsibility for the assessment and the contracting of services. Municipalities receive a flat sum from the central government (for care for older people but also for youth) and they set the tariffs for services which they contract from private providers. Private co-payments apply depending on a person's income level. The implementation of domestic care varies across municipalities as they use different assessments and conditions for providers.

Consequences of the reform

The reform aimed at strengthening home and community care as opposed to residential care as much as possible and to decentralise responsibilities to insurers and municipalities. Since 2015 individuals only qualify for residential care if they are very severely dependent and need 24-hour care, while keeping the option to receive this large amount of care also at home under the WIZ. Consequently, less dependent individuals have to stay at home and rely more on their network of family and volunteers. At the same time, the government has embarked in reforms to strengthen home care and housing support to ensure that people can receive care at home. Residential care experienced an expenditure cut of 0.5 billion from the government's decision to close nursing homes for clients with only mild limitations. Data show that the number of clients moving to nursing homes did indeed decline between 2013 and 2016 (from 48 267 in 2013 to 40 984 in 2016) (Kelders and Vaan, 2018^[16]).

This was accompanied by expenditure cuts in the WMO and ZVW (EUR 0.7 billion and EUR 0.4 billion, respectively). Under the assumption that municipalities and insurers are able to organise LTC more efficiently, these two actors are expected to increase the pressure on providers to offer better prices while also relying more on the family support. Since then, the government has provided extra investment to improve quality, to promote employment in the sector and the use of technology.

The new reform substantially decreased the funding allocated to municipalities, to the detriment of the workforce and employers. The expenditure cuts have major consequences for contracting and tariffs. Municipalities initially tried to absorb these cuts by reducing the number of hours of care that individuals were entitled to, but the Central Court of Appeals ruled that adequate and sufficient services need to be provided. Providers of domestic help reported that the budget cuts led to lower hourly rates, resulting in bankruptcies, layoffs, lower wages and short or temporary contracts. In addition, there are concerns that the focus on lower prices and costs could reduce care provision, translating into higher costs in other services i.e. faster health deterioration requiring moving to a nursing home, more hospital admissions, etc. To address this, new pricing regulations were introduced in 2017.

The division of LTC funding across three separate Acts (WLZ, Zvw and Wmo) allows for the shifting of costs among different providers. This is particularly the case for municipalities and insurers who have incentives to shift people into the public LTC scheme (Alders and Schut, 2018^[17]). Municipalities, which receive a block grant, have limited incentives to fund home support or adaptations, as their focus may shift toward encouraging individuals move into residential care instead. The varying co-payment structures across these funding schemes can encourage people to age in place, as intended. However, they may also lead to overprovision of services, because providers of nursing and personal care are paid on a fee-for-service basis. At the same time, low copayment fees for housekeeping services appear to have incentivised demand for these municipal services (Bakx, Schut and Wouterse, 2020^[18]). Since 2019, individuals' copayments for WMO have been set at a flat rate, regardless of income.

Denmark

Denmark's LTC system is among the most universal worldwide, structured through a coordinated approach across national, municipal, and provider levels. National parliamentary representatives set overarching regulations and negotiate funding agreements that shape the economic framework for LTC. Municipalities, however, hold primary responsibility for delivering both social and health-related LTC services. Within this framework, local authorities set the scope and conditions of service delivery, resulting in 98 unique municipal adaptations of the LTC system tailored to local needs (Directorate-General for Employment Social Affairs and Inclusion (European Commission), 2021).

Over the last decade, significant reforms have transformed the LTC system, impacting service delivery, eligibility, and the types of care provided. This transformation has contributed to a reduction in the number of individuals receiving LTC services, even as the ageing population grows (Rostgaard et al., 2022). The LTC system now focuses on four primary areas: preventative services, reablement, home care, and residential care. Preventative services promote health through home visits and social activities, while reablement focuses on maintaining functional abilities with medication reviews, ADL training, and assistive devices. Home care provides personal and practical support for those unable to manage daily tasks, and residential care is offered when home adaptations can no longer meet a person's needs (Directorate-General for Employment Social Affairs and Inclusion (European Commission), 2021).

While Denmark's LTC system remains universal in theory, economic and demographic pressures have led to significant reforms, particularly in the areas of service delivery and access. Denmark's LTC system has long been based on the principle of universal access, where eligibility is determined by need rather than income, assets, or family situation. However, the reforms introduced since 2008, coupled with stricter economic regulation of municipal finances, have led to de facto retrenchment in certain areas, particularly home care. This has resulted in a more restrictive interpretation of eligibility criteria for some LTC services,

effectively reducing the number of beneficiaries (Directorate-General for Employment Social Affairs and Inclusion (European Commission), 2021; Rostgaard et al., 2022).

For example, while access to home care has not been explicitly limited by new policies, municipalities have made eligibility assessments stricter, leading to a reduction in the number of people qualifying for home care. Between 2016 and 2018, the total weekly home care hours provided to individuals over 65 decreased by 6.4%, with 1.5 million fewer hours of care being delivered annually. During the same period, the number of people receiving home care services decreased slightly by 0.7%, despite a growing elderly population (Directorate-General for Employment Social Affairs and Inclusion (European Commission), 2021). Cuts to home care services have also impacted specific support areas, such as cleaning services, which have been significantly reduced since 2008. Visits now occur as infrequently as once a month, compared to weekly in the past, making these services more symbolic than practical for many beneficiaries (Rostgaard et al., 2022).

The integration of reablement programmes have also reshaped the LTC landscape by focusing on maintaining or restoring autonomy. Introduced locally in 2007 and formally included in the service act governing home care in 2015, reablement requires municipalities to assess whether an older person has the potential to benefit from short-term, interdisciplinary interventions (typically lasting up to 12 weeks) before regular home care is offered. These programs aim to enhance daily functioning according to each client's individual preferences and goals. With the introduction of reablement, overall coverage and hours of home care have been significantly reduced in recent years (Directorate-General for Employment Social Affairs and Inclusion (European Commission), 2021; "Reablement in Long-Term Care for Older People: International Perspectives and Future Directions," 2023; Rostgaard et al., 2022). In 2018, 4.3% of people aged 65 years and over received reablement services instead of – or alongside – home help (Rostgaard, 2021_[19]). Danish evidence suggests improvement in functional ability, better evaluation of working conditions and motivation among staff. Local reports indicate good user outcomes, and some studies show a lower use of home care (Rostgaard, 2024_[20]). The probability of receiving home care for frail people aged 67-87 years decreased from over 35% to about 25% thanks to the use of reablement (Rostgaard, 2021_[19]).

The economic impact of reablement has been mixed. Four studies examining the cost-effectiveness of reablement programs found variable outcomes (Kjellberg & Ibsen, 2010, 2012, 2016b, 2016a). Two studies showed increased costs and hours in regular home care services, suggesting that reablement may raise overall expenditures as some clients continue to need substantial support post-intervention (Kjellberg & Ibsen, 2016b, 2016a). One study found reablement to be cost-neutral, with reductions in service demand offset by the investment needed to implement these programs within regular home care (Kjellberg & Ibsen, 2010). Another study found an increase in costs, underscoring that reablement may not always reduce expenses but can shift resources toward targeted, short-term interventions that potentially extend clients' autonomy (Kjellberg & Ibsen, 2016b).

In conclusion, while Denmark's LTC system maintains its foundational principles of universal eligibility, stricter assessments and a growing emphasis on reablement have effectively narrowed access to certain services, particularly home care. These changes reflect a nuanced balancing act as Denmark addresses demographic and economic challenges. The introduction of reablement programmes demonstrates a forward-looking approach to preserving autonomy for older adults; however, the cost-effectiveness of these programmes remains variable, with financial impacts still under evaluation.

New Zealand

The consistent application of needs assessment has contributed to shifting formal care towards those with severe needs. The share of those aged 65 years and over who received public LTC benefits considerably falling by 6.6 percentage points to 11.4% over the nine years to 2020. In New Zealand, the 20 regional District Health Boards (DHBs) had undertaken needs assessment by holistically evaluating

limitations on ADLs, IADLs, mental health, physical, social and cognitive functioning, as well as the presence of informal carers (Ministry of Health and NZGG, 2003^[21]; Ministry of Health, 2011^[22]).

Preliminary analyses also suggest that the Pay Equity Settlement (PES) Act of 2017 might have contributed to this decline by reducing the supply of formal care. Although the PES successfully increased wages for care workers and addressed pay disparities, some reports proposed that employers may have rationed working hours to manage rising staffing costs. However, subsequent studies examining the PES impact indicate no definitive evidence of reduced working hours for care workers, suggesting that this alone is unlikely to account for the observed decrease in LTC beneficiaries.

Further analysis considered other policy initiatives, including the Better Later Life – He Oranga Kaumātua 2019-2034 strategy, the Healthy Ageing Strategy 2016, the New Zealand Disability Strategy 2016, and the New Zealand Carers' Strategy 2008. Although these policies do not directly alter LTC eligibility or benefit coverage, they emphasise supporting older adults in ageing within their communities, promoting independence, and reducing institutional care reliance. This shift may indirectly contribute to lower enrolment in formal, publicly funded LTC services, as more older adults opt for community-based and informal care solutions.

It is also relevant to explore whether an increase in informal caregiving has influenced the trend, with policies such as carer subsidies and supported living payments potentially incentivizing family-led care. Further clarification are anticipated to refine these insights, and an updated analysis will be included in the following report.

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